

Integration of traditional, complementary and integrative medicine into health systems

Conceptual framework



World Health
Organization

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ISBN 978-92-4-011039-7 (electronic version)

ISBN 978-92-4-011040-3 (print version)

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Cataloguing-in-Publication (CIP) data. CIP data are available at <https://iris.who.int/>.

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Design and layout by Inis Communication

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Preface

The WHO Global Traditional Medicine Strategy 2025–2034, adopted by the Seventy-eighth World Health Assembly, provides a ten-year roadmap to harness the evidence-based, safe, and effective contribution of traditional, complementary and integrative medicine (TCIM) towards health and well-being for all. Building on Member States’ long-standing commitments to the integration of TCIM—from the Alma-Ata Declaration on Primary Health Care in 1978 to the 2023 UN General Assembly Political Declaration on Universal Health Coverage—the Global Strategy sets out four strategic objectives: to strengthen TCIM evidence, regulation, integration and collaboration. These objectives also align with WHO’s Fourteenth General Programme of Work (GPW 14), which envisions resilient, equitable and people-centred health systems that promote, provide and protect the health and well-being of all people, everywhere.

Within this vision, the WHO Global Traditional Medicine Centre (TMC), as part of the Health Systems, Data and Access Division (HSD), contributes to advancing the integration of TCIM within countries’ health systems architecture through research and innovation, norms and standards, and partnerships and sustainability. This Conceptual Framework for the Integration of TCIM into Health Systems provides countries with practical approaches to define, compare and strengthen TCIM integration across health systems building blocks, and tailor integration strategies to unique country contexts.

Developed through evidence reviews and broad consultation, this conceptual framework reflects WHO’s commitment to context-specific, evidence-based and mutually-respectful integration of biomedicine and TCIM to strengthen health systems and quality of care. Looking ahead, TCIM’s holistic philosophies and system-based approaches can inform emerging global trends towards more personalized, preventive and sustainable models for health and well-being of people and planet.



Dr Shyama Kuruville
Director a.i.
Global Traditional Medicine Centre



Dr Rudi Eggers
Former Director
Integrated Health Services

Acknowledgements

WHO conveys its sincere appreciation to all individuals and organizations that contributed to the development of this document.

WHO is grateful to all Member States that provided support and collaboration for its development.

Special thanks are extended to Heather Boon and her colleagues at the University of Toronto, Canada, for their substantive contributions to the evidence review and the drafting of the document.

WHO recognizes the valuable contributions of Peter Bai James, Rachel Canaway, Vinjar Fønnebø, Daniel Gallego, Ossy Muganga Julius Kasilo, Mohammed Khalil, Werner Knoss, Rajeshwari Singh, Sara Warber, and Zhang Qi, who provided expert advice and constructive inputs during the working group meeting, peer review, and consultations.

Liu Qin and Aditi Sharan coordinated the overall development process to ensure technical coherence and collaboration across all stages, under the guidance of Kim Sungchol, Rudi Eggers, and Shyama Kuruvilla.

WHO acknowledges the contributions of colleagues across headquarters, regional, and country offices for their technical and administrative support.

WHO gratefully acknowledges the Government of the Macao Special Administrative Region of China (Macao SAR, China) for its financial support for the development of this document and for hosting the WHO working group meeting.

Glossary

Biomedicine: also referred to as “conventional medicine” (1) and includes reference to “biomedical”, is a branch of medical science in which biological and physiological principles are applied in clinical practice. Biomedicine stresses standardized, evidence-based treatment validated in biological research, with treatment administered by formally trained doctors, nurses and other licensed practitioners (2).

Traditional medicine: refers to codified or non-codified systems for healthcare and well-being comprising practices, skills, knowledge and philosophies originating in different historical, cultural contexts, that are distinct from and pre-date biomedicine, evolving scientifically for current use from an experience-based origin. It emphasizes nature-based remedies and holistic, personalized approaches to restore balance of mind, body and environment (3, 7).

Complementary medicine: refers to additional healthcare practices that are not part of a country’s mainstream medicine. Evidence-based complementary medicine has the potential to support mainstream medicine and more comprehensively support people’s health and well-being needs.

Health products: health technology or device used in the prevention, diagnostics, treatment, rehabilitation or palliation of diseases. Include medicines, vaccines, medical devices, in vitro diagnostics, protective equipment, assistive devices and vector control tools (4).

Health service: Any service (not limited to medical or clinical services) for contributing to improve health or to the diagnosis, treatment and rehabilitation of individuals and populations (5).

Health system: Systems that comprise all the organizations, institutions and resources devoted to providing health actions, defined as any effort, whether in personal health care, public health services or through intersectoral initiatives, the primary purpose of which is to improve health (6).

Integrative medicine: refers to an interdisciplinary and evidence-based approach to health and well-being by using a combination of biomedical and traditional and/or complementary medical knowledge, skills and practices (7).

Traditional, complementary and integrative medicine (TCIM): a merging of the terms “traditional medicine”, “complementary medicine” and “integrative medicine” (7).

1 Introduction

Traditional, complementary and integrative medicine (TCIM), defined in the glossary, is present in almost every country in the world, and the demand for its services is increasing (3,8). Several World Health Assembly resolutions have encouraged Member States to explore how they could contribute to strengthening integrated, people-centred health services. The World Health Assembly has also requested WHO to provide technical support and guidance to Member States in implementation, national adaptation and operationalization of a framework for integrated, people-centred health services (9). See Annex 1 for a complete list of WHO mandates on TCIM. In United Nations General Assembly resolution 78/4 (2023) on universal health coverage (10), heads of state and government, in recommitting themselves to achieve universal health coverage by 2030, committed to, inter alia, explore ways to integrate, as appropriate, safe and evidence-based traditional and complementary services within national and local health systems, particularly at the level of primary health care, according to national context and priorities.

The WHO Global Traditional Medicine Strategy 2025–2034, adopted by the Seventy-eighth World Health Assembly, builds on this commitment by providing a ten-year road map for harnessing the contribution of traditional medicine to health and well-being through strengthened evidence, regulation, system integration and cross-sector collaboration. Integrating safe and effective TCIM into health systems has been identified as one of the strategic objectives for the next decade.

Countries continue to recognize the need to develop a cohesive, integrative approach to health care that allows governments, health-care practitioners and, most importantly, those who use health services, to access TCIM in a safe, respectful, cost-efficient, effective manner (11).

Each Member State decides whether and how to include TCIM in their health systems. The decision may be based on a number of factors:

- the types, number and diversity of supply of TCIM and biomedicine practitioners, products and services in the country;
- population demand according to the country's cultural traditions; and
- the knowledge base on the clinical effectiveness and safety of use of TCIM products and services.

Integration of TCIM into their health systems may be a means for countries to achieve the third SDG to “Ensure healthy lives and promote well-being for all at all ages” (12). Some models of integration may be more effective than others for achieving SDG3. One challenge to providing guidance on the “best” integration model(s) is the lack of consistent terminology to describe different models of integration used by countries around the world. An important goal of this document is to provide a framework of models of integration to facilitate discussion, comparison and research and to enable the identification and implementation of effective integration of TCIM.

This document marks an initial step to support TCIM integration aligned with countries' current health systems. Going forward, TCIM, through its inherently holistic philosophies and system frameworks, can help shape emerging global trends towards more personalized, preventive, and sustainable approaches to the health and well-being of people and the planet.

1.1 Document development and consultation

In accordance with the WHO process for document development, WHO assembled and led a team of several global experts with proven clinical, research and policy-making experience in the field of TCIM. With its country offices and in consultation with regional advisers on TCIM, WHO requested several Member States to share relevant information that would allow WHO to understand the current role and position of TCIM in their health systems (see Annex 5).

WHO received responses from 20 Member States, and their national policies and information materials were used to prepare the zero draft. After a review of the responses and in consultation with WHO country offices, on-site visits were conducted to observe the realities of TCIM integration in those countries. The first draft of this guidance was then prepared.

The patterns of use of TCIM presented in the WHO Traditional Medicine Strategy 2014–2023 (3) were used to identify Member States that represented a range of approaches to integration of TCIM into their health systems. The first pattern of use of TCIM was found in countries in which TCIM is a primary source of health care, in the African Region, where traditional medicine is firmly rooted, although there is limited formal documentation and education. From information shared by Member States in WHO global report on traditional and complementary medicine 2019 and in discussion with the Regional Advisor for Traditional Medicine, Ghana, Madagascar, Mali and South Africa were studied.

The second category of countries was in which TCIM is widespread because of cultural and historical influences but in which there is still scope for integration at systemic level. These included countries in the South-East Asia and Western Pacific regions, where, for example, the Ayurveda system of traditional medicine in India and traditional Chinese medicine in China are globally practised systems of traditional medicine (3). China, India, Japan, Republic of Korea and Thailand, with the largest populations, were approached to identify best practices and areas in which there could be improvement.

The third pattern of TCIM use was identified in countries in which the structure of the health system is usually well developed, TCIM originates mainly from the outside and which had attempted to integrate complementary medicine into their health systems. Case studies included Brazil, Norway and Portugal.

The purpose of the site visits was to obtain an overview of the context of traditional and complementary medicine in the country, including policy and regulation; TCIM products, practices and practitioners; TCIM education, research and service delivery; and integration of TCIM into the national health system. During the site visits, meetings were held with senior policy-makers, institutional managers, doctors, academic faculty and people seeking care, and visits were made to various health-care institutions, such as biomedical and traditional medical universities and hospitals, herbal medicine manufacturing facilities and primary health care centres. During the visits, the experts and WHO observed the levels of service delivery, the educational systems and research support for biomedicine and for TCIM.

In November 2019, with the support of the Government of Macao Special Administrative Region of China (Macao SAR, China), a global expert consultation workshop was conducted to discuss and review the draft document.

The document was further revised after the workshop and also according to the findings of an evidence review to identify key concepts and research on integration of TCIM into health systems. Six electronic databases were searched with no date or language limits. This was supplemented by a search of the grey literature and a review of the reference lists of studies that met the inclusion criteria. Screening was conducted in duplicate by four independent reviewers.

Best Fit Framework Analysis with NVivo was used to synthesize the qualitative evidence, with the WHO Health System Building Blocks as the theoretical framework. A total of 25 512 articles were identified. After screening, 53 articles written by authors based in all six WHO regions and published between 1976 and 2023 were found to meet the inclusion criteria. The PRISMA-ScR checklist was used to report the findings (see Annex 7).

The revised document was reviewed in a global peer review and several small expert consultations. A second global expert consultation was conducted virtually in August 2024 (see Annex 6).

The four objectives of this document are to:

- provide Member States with a WHO definition of “integration” of TCIM into health systems;
- provide a framework for comparison and assessment of models of integration of TCIM into health systems;
- provide an assessment tool for determining the model of integration of TCIM to be used in a health system; and
- identify characteristics of evidence-based models of integration of TCIM into health systems.

1.2 Organization of the document

This document begins with a definition of “integration of TCIM into health systems”. Section 3 introduces a conceptual framework for describing and comparing models of integration in two dimensions: decentralized versus centralized and pluralistic versus unification. With these two dimensions, four categories of models of integration are proposed: people-led (decentralized, pluralistic), category A; practitioner-led (decentralized, unification), category B; a coordinated system (centralized, pluralistic), category C; and a blended system (centralized, unification), category D. This is followed by an exploration of how the WHO health system building blocks (leadership and governance; health system financing; health service delivery; health workforce; health information systems; health products; and vaccines and technologies) provide guidance with respect to the data to be collected to summarize a country’s integration model and to categorize it according to the framework. The characteristics of effective models of integration are described, so that countries can identify elements on which they may wish to focus to enhance integration of TCIM into their countries’ health systems.

2 Definition of integration of TCIM into health systems

A WHO technical brief that addressed the meaning of “integrated health services” (13) provided a working definition of integrated service delivery as follows:

the management and delivery of health services so that people receive a continuum of preventive and curative services, according to their needs over time and across different levels of the health system.

The Sixty-ninth World Health Assembly (9) further defined integrated health services as health services

that are managed and delivered so that people receive a continuum of health promotion, disease prevention, diagnosis, treatment, disease-management, rehabilitation and palliative care services, coordinated across the different levels and sites of care within and beyond the health sector, and according to their needs throughout the life course.

Neither definition explicitly refers to integration of TCIM into a health system. Therefore, the following definition is proposed:

the organization, management and financing of the health system to incorporate the equitable delivery of accessible, culturally appropriate, people-centred, evidence-based traditional, complementary and biomedical health services and products across the continuum of care in the pursuit of optimal health and well-being for all.

Given the widespread use of TCIM by people worldwide and the definition above, a health system into which TCIM is integrated could assist countries in achieving the third United Nations SDG (12) “to ensure healthy lives and promote well-being for all at all ages”. WHO acknowledges that there has been concern about the possibility of co-option or misappropriation of practices or entire systems or disciplines by biomedicine. For example, Indigenous peoples have raised concern about the risks that integration might pose for their intellectual property rights and the benefit-sharing of their traditional medical knowledge. Integration of TCIM into health systems as described in this document requires respectful collaboration among practitioners and knowledge holders in recognition of all rights and privileges granted in the United Nations Declaration on the Rights of Indigenous Peoples (14) and all other related international instruments, including the Indigenous and Tribal Peoples Convention (No. 169 of 1989) (15), the Convention on Biological Diversity (1992) (16) and the Treaty on Intellectual Property, Genetic Resources and Associated Traditional Knowledge (2024) (17). Terms such as “coordination” and “articulation” are also used to describe such integration; for simplicity, the term “integration” is used throughout this document.

The conceptual framework described below is based directly on a report by Rachel Canaway (18) for the WHO Regional Office for South-East Asia and further development of the concept of integration of TCIM into national health-care systems by Park and Canaway (19). See Annex 2 for their conceptual map of how TCIM integration is influenced by country-specific contextual factors. Park and Canaway note that any model of TCIM integration into health systems is

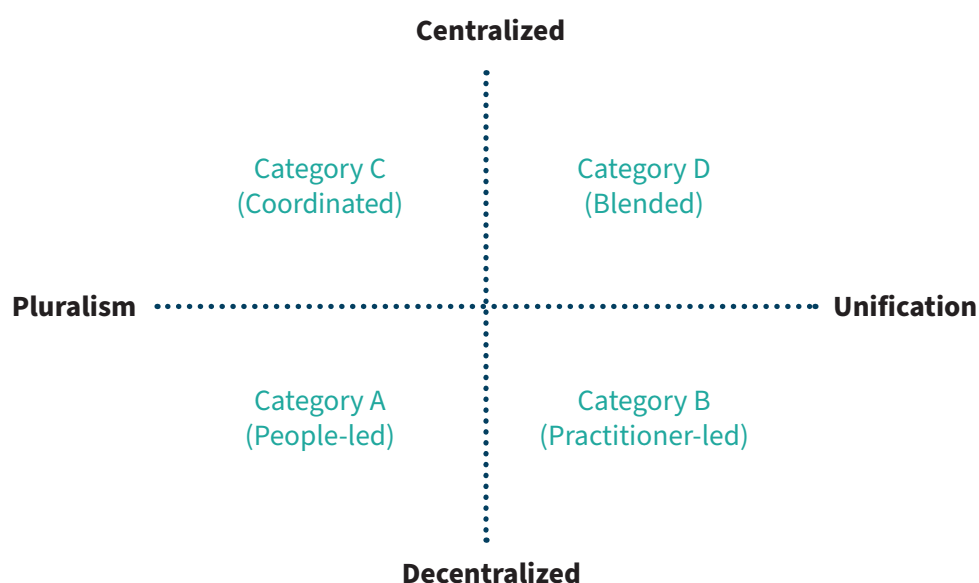
affected in various ways, such as the setting (e.g. natural environment, population density and demographics, socio-cultural ideology) and the current health-care system (e.g. system, health governance and financing, health service delivery, health workforce, service standards and health information).

3 Conceptual framework: Categorizing models of integration of TCIM into health systems

As noted above, each Member State decides whether and how to include TCIM in their health systems. One challenge to integration of biomedicine and TCIM is that the systems may be rooted in paradigms with a different underlying ontology and epistemology. An important consideration for integration is how different ways of knowing and different forms of evidence can be acknowledged and used in an integrated health system.

There is no single “best” model for integration of TCIM with biomedicine. This conceptual framework groups models for integrating TCIM and biomedicine into four categories according to two dimensions. One dimension that can be used to differentiate approaches is whether a decentralized or a centralized approach is used. The people-led (category A) and practitioner-led (category B) models represent decentralized approaches, in which integration is led by people in the community and practitioners, respectively. In contrast, coordinated (category C) and blended (category D) models are based on a centralized approach characterized by government or system-level decision(s) requiring or planning to integrate TCIM into the country’s health systems (Fig. 1).

Fig. 1 Conceptual framework categorizing models of integration



A second dimension that can be used to differentiate approaches to integration is whether the model calls for pluralism or unification of TCIM and biomedicine. Pluralism is generally characterized by systems in which TCIM and biomedicine preserve their unique identities, approaches and contributions to health. A concept inherent to pluralism (sometimes called therapeutic or medical pluralism) is that its goal is to facilitate a choice of the most suitable treatment option(s). In pluralistic systems, people's autonomy is prioritized, and the integrity of the different treatment systems is valued and supported (20,21). In contrast, the goal of unification is to combine the components of individual approaches into a single new health-care system to enable better health outcomes. A justification of unification is that it results in a new type of health system that is “greater than the sum of its parts”.

With these two dimensions, integration models can be grouped into four categories:

- people-led, which is decentralized pluralistic (category A);
- practitioner-led, which is decentralized unification (category B);
- coordinated, which is centralized pluralistic (category C); and
- blended, which is centralized unification (category D)

Integration models that fit into the people-led (A) or coordinated (C) categories are generally pluralistic. In people-led models, people coordinate or combine support from the various therapeutic approaches. In coordinated (category C) models, the expressed goal of the health system is to coordinate distinct but equitably supported and accessible TCIM and biomedical services.

A basic concept of the practitioner-led (category B) and blended (category D) integration models is unification of the paradigms, philosophies and understanding of “evidence” for TCIM with those of biomedical approaches to create a new health system. Unification has been considered by some to be the only model that achieves “full” or “total” integration of TCIM (e.g. 20–26). Witt et al. (25) described this integration model as the “best of both worlds” and noted that it requires development of a “new”, shared culture. Coulter (23) described it as a metaparadigm. In practitioner-led models (category B), individual practitioners lead integration by using the approaches of their practices or by overseeing teams of practitioners who work together to unify their approaches. In blended (category D) integration models, unification is mandated and organized within the health system.

In this framework, there is no inherently “best” model of integrating TCIM into health systems. The ways in which various countries enact integration are described. Guidance on the characteristics of effective approaches that may be used in any of the four models in the framework is provided in section 5. It is not expected that every country will adopt one of the four categories. Some will not integrate TCIM into their health systems, and some do not fit into any category. This framework is designed only for those countries that wish to integrate TCIM into their health systems in some way.

3.1 People-led (category A) models

People-led models of integration are decentralized pluralistic models, in which people choose or decide on the therapies or therapeutic approaches they wish to integrate into their individual care plans. There may be government support for preservation of the TCIM culture and use of TCIM may be high; however, there is limited government direction or guidance on integrating TCIM into the country's health systems. Government regulation in people-led models is primarily to ensure the availability (and accessibility) of a wide range of both TCIM and biomedical products and services that are safe for patients to integrate in their care plans. A key strength of people-led models is the people-centred approach, which empowers individual choice and tailored approaches to care. The burden of constructing an integrated approach to care, however, rests with individuals.

3.2 Practitioner-led (category B) models

Practitioner-led models of integration are decentralized unification models. The focus is on integration of decision-making and blending of different therapeutic approaches by individual practitioners or small groups of practitioners working together in a clinical setting. The two main types of practitioner-led models are:

- practitioners who integrate biomedicine and TCIM by practising both themselves; and
- biomedicine practitioners who integrate TCIM into care plans or TCIM practitioners who integrate biomedicine into care plans by referring to and/or collaborating with other practitioners.

A strength of practitioner-led models is that both TCIM and biomedical practitioners with appropriate expertise have the flexibility to blend approaches to care from various health-care approaches to create a unified care plan for people under their care. A potential challenge to these models is wide variation in the care experienced from different practitioners and difficulty in assessing the quality of the blended care.

3.3 Coordinated (category C) models

Coordinated models of integration are centralized pluralistic models. They can be implemented in many ways, but all focus on facilitating coordination of TCIM and biomedical health services to provide a better experience (and ideally better health outcomes) for people while retaining the distinct biomedical and TCIM health-care practices to ensure the unique contributions of both. An advantage of coordinated models is the support of structures that facilitate formal coordination of services and approaches and may enhance access to a more uniform quality of care to more people. A challenge of these models is ensuring adequate support and engaging both TCIM and biomedical practitioners in developing mutual respect and willingness to collaborate.

3.4 Blended (category D) models

Blended models are centralized unification models. They rely on definition or mandating by leadership of integration of TCIM as part of the country's health systems and blending of cultures, systems and approaches to create a new health system that combines both biomedicine and TCIM. An advantage of blended models is that there is only "one" health systems, which simplifies access. A challenge of these models is that blending health-care services and approaches from systems of care with vastly different ontological and epidemiological bases may be difficult and may be resisted by both TCIM and biomedical practitioners.

3.5 Further considerations on the conceptual framework

The conceptual framework is an approach to understanding different approaches to integration of TCIM and biomedicine into health systems. No integration is another possibility, which is outside the framework. The four categories of model that make up the framework are deliberately presented as four quadrants defined by two dimensions (decentralized versus centralized; pluralism versus unification) (Fig. 1), so as not indicate (or imply) a value judgement about the different categories. All four categories are valid approaches to integration of TCIM into health systems, which may be effective in different country contexts. The conceptual framework should not be interpreted to imply that countries are expected to evolve from category A through to categories B, C and D. No hierarchy or linearity is associated with this framework. Countries that choose any of the four integration models may decide to continue with one model or to evolve towards another. Section 4 provides additional guidance for countries on identifying how their current model fits into the conceptual framework.

4 Conceptual framework for integration and health system building blocks

This section outlines how the conceptual framework can be applied to a country's health system. The building blocks of health systems (27) provide a starting point for categorizing countries' approaches to integration of TCIM into their health systems. The six discrete "building blocks" that make up the health system provide common understanding of the desirable attributes of health systems and are useful for clarifying the essential functions of health systems (28). They are:

- leadership and governance;
- health system financing;
- health service delivery;
- health workforce;
- health information systems; and
- health products, vaccines and technologies.

Categorization requires a review of if and how TCIM is included with respect to all six building blocks. The current integration models of many countries will not fit neatly into a single category. Identification of the category into which each building block falls, however, will provide a holistic picture of the country's integration model, which can be used to catalyse discussions on the type of integration that is desirable for the country's context. The framework is also useful for comparing the health systems of different countries and can be used to document changes over time in a country.

The following descriptions are based on findings from the literature review and the site visits. They represent examples of current health systems and are not recommendations.

4.1 Leadership and governance

People-led (category A) models

- Access to TCIM is accepted and is considered an individual right.
- TCIM is acknowledged as a resource in national health planning.
- There are few dedicated institutions or policies for TCIM (i.e. generally no dedicated ministry, office, centre or regulations).
- There is limited government oversight or accountability with respect to the availability, accessibility, quality or effectiveness of TCIM.
- There is some national oversight of the safety of TCIM.

Practitioner-led (category B) models

- Access to TCIM is accepted and is considered an individual right.
- TCIM is acknowledged as a resource in national health planning.

- There are few dedicated institutions or policies for TCIM (i.e. generally no dedicated ministry, office, centre or regulations).
- There is limited government oversight or accountability with respect to the availability, accessibility, quality or effectiveness of TCIM.
- Regulation or government oversight allows individual practitioners to integrate TCIM practices with biomedical practices and/or allows TCIM and biomedical practitioners to collaborate.
- There is some national oversight of the safety of TCIM.

Coordinated (category C) models

- Access to use of TCIM health products and services is recognized and supported by the government as a distinct part of the health system, often through a specific policy statement.
- TCIM is included as a distinct component of national health planning.
- There is commonly a distinct national TCIM ministry, department or office.
- There is government oversight or accountability with respect to the safety, availability, accessibility, quality and/or effectiveness of TCIM (respecting the unique qualities of TCIM).

Blended (category D) models

- Government policy has established the integration of TCIM and biomedicine into a single, unified health system.
- National health planning includes both TCIM and biomedicine.
- TCIM and biomedicine are often blended in relevant ministries, departments or offices.
- There is government oversight or accountability with respect to the safety, availability, accessibility, quality and/or effectiveness of health interventions of both TCIM and biomedicine.

4.2 Health system financing

People-led (category A) models

- There is limited core health funding for TCIM services or products.
- TCIM funding may be available as a supplementary package in a national health insurance scheme for individuals who desire such coverage.
- TCIM funding is available from philanthropic organizations, private insurance and out-of-pocket payments.

Practitioner-led (category B) models

- There is limited core health funding for TCIM services or products.
- TCIM funding may be available as a supplementary package in a national insurance scheme for individuals who desire such coverage.
- Individual practitioners may be funded by national health insurance schemes to provide some TCIM services and products as part of their blended care options.
- TCIM funding is available from philanthropic organizations, private insurance and out-of-pocket payments.

Coordinated (category C) models

- Core health funding is available specifically for TCIM services and products.
- Distinct, equitable funding designated for TCIM is included in a national health insurance scheme.
- TCIM funding is available from philanthropic organizations, private insurance and out-of-pocket payments.

Blended (category D) models

- Core health funding includes both TCIM and biomedical services and products as part of a unified system.
- Equitable funding is included for blended TCIM and biomedical provider services and products in a national health insurance scheme.
- Funding is available for products and services as part of a unified system from philanthropic organizations, private insurance and out-of-pocket payments.

4.3 Health service delivery

People-led (category A) models

- People have access to health services delivered by two distinct groups of practitioners: TCIM and biomedicine.
- TCIM practitioners provide preventive, curative, rehabilitative and palliative care.
- There is limited direct communication or collaboration between TCIM and biomedical practitioners; individuals facilitate communication among members of their health-care team.
- Individuals coordinate their own integrated care.

Practitioner-led (category B) models

- People have access to TCIM health services from both TCIM and biomedical practitioners.
- TCIM and biomedicine practitioners voluntarily create practitioner teams that provide preventive, curative, rehabilitative and palliative care.
- Biomedical practitioners voluntarily offer TCIM health services, and TCIM practitioners voluntarily offer biomedical health services.
- Some providers of health care, including some TCIM practitioners, communicate and collaborate with each other, patients and families, primarily in practitioner-led integrative clinics.
- People receive coordinated care in practitioner-led integrative clinics.

Coordinated (category C) models

- People have access to distinct but coordinated health services delivered by two distinct groups of practitioners: TCIM and biomedicine.
- TCIM practitioners provide preventive, curative, rehabilitative and palliative care.
- The national health system supports coordination of the distinct services of all health-care providers, including TCIM practitioners.
- Providers of health care, including TCIM practitioners, communicate and collaborate with each other, patients and families to facilitate coordination of their distinct services.
- People experience coordinated care from a wide range of health practitioners, including TCIM practitioners.

Blended (category D) models

- People have access to health services from both TCIM and biomedical practitioners that is blended to provide preventive, curative, rehabilitative and palliative care.
- The health system infrastructure includes blending of the services of all health-care providers, including TCIM practitioners, and fills administrative and programmatic needs at all levels in a wide range of settings.
- Providers of health care, including TCIM practitioners, communicate and collaborate with each other, patients and families (often in interdisciplinary teams) to provide blended care.

- People experience seamless care from a wide range of health practitioners, including TCIM practitioners.

4.4 Health workforce

People-led (category A) models

- People have access to a practitioner workforce composed of two distinct, separate groups: biomedical and TCIM health-care practitioners .
- TCIM practice and education are generally not regulated by the government; safety standards are generally in place.
- Practitioners organize and monitor themselves, including with respect to education, quality assurance and forms of certification.
- Some funding may be available for the education of TCIM practitioners.
- TCIM practitioners and biomedical providers uphold their own separate, individual ethical standards.

Practitioner-led (category B) models

- People have access to a practitioner workforce composed of biomedical practitioners and TCIM practitioners; some TCIM practitioners work in practitioner-led integrative clinics, some biomedicine practitioners offer TCIM services, and some TCIM practitioners offer biomedicine services.
- Regulation or oversight of the practice and education of biomedical providers facilitates blending of TCIM practices into biomedicine practice by individual practitioners or by collaboration with TCIM practitioners.
- Regulation or oversight of the practice and education of TCIM providers facilitates blending of biomedical practices with TCIM practice by individual practitioners or through collaboration with biomedical practitioners.
- Practitioners generally organize and monitor themselves, including with respect to education, quality assurance and forms of certification.
- Some funding may be available for biomedical practitioners to learn some TCIM practices or for TCIM practitioners to learn some biomedical practices.
- TCIM and biomedical practitioners who work together in practitioner-led integrative clinics voluntarily uphold the same practice and ethical standards.

Coordinated (category C) models

- People have access to a practitioner workforce composed of two distinct, separate groups: biomedical health care practitioners and TCIM practitioners.
- TCIM practice and education are appropriately regulated by the government in a manner distinct from biomedical practice and education.
- The education of all health-care practitioners, including TCIM practitioners, includes the information and skills necessary to coordinate health services and care with other providers.
- Some funding may be available for biomedical practitioners to learn about TCIM practices or for TCIM practitioners to learn about biomedical practices.
- TCIM practitioners and biomedical providers uphold their own separate, appropriate ethical standards.

Blended (category D) models

- People have access to a blended practitioner workforce composed of biomedical and TCIM practitioners.

- TCIM and biomedical practice and education are appropriately regulated by the government within a single regulatory framework.
- The education of all health-care practitioners, including TCIM practitioners, provides the common knowledge and skills necessary to blend their health services and care with those of other providers.
- Equitable funding is dispersed by similar mechanisms for the education of both TCIM and biomedical practitioners.
- TCIM practitioners and biomedicine providers uphold common ethical standards.

4.5 Health information systems

People-led (category A) models

- Providers of health care, including TCIM practitioners, document their practices in individual ways and voluntarily share information with others.
- There are few health system structures for sharing data, communication or collaboration between TCIM and other health-care practitioners.
- Billing, charting and referral health systems (when present) generally do not include sharing of health information between TCIM and biomedical practitioners.

Practitioner-led (category B) models

- Providers of health care, including TCIM practitioners, document their practices in a manner that facilitates communication and collaboration with other practitioners in a practitioner-led integrative clinic or service.
- Structures to support voluntary sharing of data, communication and collaboration between TCIM practitioners and biomedicine practitioners, patients, families and communities have been created within individual practitioner-led integrative clinics or services.
- Billing, charting and referral health systems include sharing of health information with TCIM practitioners in individual practitioner-led integrative clinics or services.

Coordinated (category C) models

- Providers of health care, including TCIM practitioners, document their distinct practices in a patient record system in a manner that facilitates communication and coordination with other practitioner groups.
- There are structures to support sharing of data, communication and coordination of the distinct services of TCIM practitioners with other health-care practitioners, patients, families and communities.
- Billing, charting and referral health systems include sharing of health information that supports the distinct practices of health-care providers, including TCIM practitioners.

Blended (category D) models

- A system exists to facilitate sharing of clinical, education and research information.
- Providers of health care, including TCIM practitioners, document their practices in a manner that facilitates communication and collaboration with all practitioner groups.
- Structures are in place to support sharing of data, communication and collaboration among all health-care providers within a single system, including TCIM practitioners, patients, families and communities.
- Billing, charting and referral health systems include sharing of health information to support the blended practices of all health-care providers, including TCIM practitioners, equitably.

4.6 Health products, vaccines and technologies

People-led (category A) models

- People have access to a wide range of TCIM and biomedical health products.
- The quality, safety and effectiveness of TCIM products are assessed primarily by individuals and their TCIM practitioners.
- There is limited government regulation of TCIM health products; regulation is in place to ensure safety.

Practitioner-led (category B) models

- People have access to a wide range of TCIM and biomedical health products.
- Both TCIM and biomedical health-care practitioners guide and support people in identifying TCIM products that are of appropriate quality, safety and effectiveness.
- There is limited government regulation of TCIM health products; regulation is in place to ensure safety.

Coordinated (category C) models

- People have access to a wide range of TCIM and biomedical health products.
- Health products, including TCIM health products, are regulated; TCIM health products are recognized as distinct from biomedical products in the regulation.
- Government regulation enables people and practitioners to identify TCIM health products that are of high quality, safe, effective and cost-effective.

Blended (category D) models

- People have access to a wide range of TCIM and biomedical health products.
- Health products, including TCIM health products, are regulated.
- TCIM health products and biomedical products are appropriately regulated in the same regulatory structure.
- Government regulation enables people to identify TCIM health products that are of high quality, safe, effective and cost-effective.

4.7 Using the conceptual framework and the health system building blocks

The conceptual framework is useful for categorizing a country's model of integrating TCIM into the health system. Each country, however, may use several approaches for each health system building block, for different practitioner groups and/or for different sites of care. Recognition of this complexity is important in using the framework to categorize countries' health systems. Data routinely collected by WHO on countries' approaches to TCIM may make it possible identify how a country's model of integration of TCIM into their health systems fits into the conceptual framework. WHO is currently pilot-testing an assessment tool that may facilitate such categorization. Annex 3 outlines the integration assessment tool (IAT) that is currently under consultation. Annex 4 provides three country examples of how the framework might be used.

5 Considerations for effective integration of TCIM into health systems

As people's needs and current health systems in countries around the globe differ markedly, a "one size fits all" concept of "good" integration is neither possible nor useful. A framework for comparing different approaches to integration can, however, facilitate debate on the pros and cons of different models. It also provides a common language and common definitions to facilitate research on the different models, recognizing that the "best" model is likely to depend on the context. Thus, the best integration model for one country may be different from that for another.

Evidence collected during the country site visits and the literature review showed that one of the most important considerations in integrating TCIM into health systems is mutual respect and collaboration among providers of TCIM and of biomedicine. Health providers in an integrated health system must learn to work together with a shared goal to provide people-centred (rather than system-centred) care. Other considerations fundamental to successful integration include ensuring access to both TCIM and biomedical products and health services that are safe, effective, efficient and sufficiently resourced. Research with methods that embrace the complexity of TCIM interventions should be conducted to build such an evidence base. In addition, a mechanism should be found to ensure appropriate standards of education for both TCIM and biomedical practitioners with the expected levels of knowledge, skill and attitude.

The evidence from the country site visits and the literature also indicates that some level of governance or leadership is required for effective integration of TCIM into health systems in a centralized approach. This does not necessarily mean that countries should adopt the coordinated (category C) or blended (category D) model. Rather, it indicates that all integration models require support from leadership to be effective. For example, integration in all model categories requires a policy environment and leadership decisions to facilitate the availability of safe, effective, high-quality, accessible health products and health interventions provided by appropriately qualified practitioners.

The evidence acquired also does not indicate that pluralism or unification is inherently better. Thus, it is not clear whether the patient-led (A) and practitioner-led (C) categories are inherently better than the coordinated (B) and blended (D) categories of integration for achieving good integration of TCIM into health systems. Many definitions of "integration" in the literature indicate that blended models are the ideal type of integration, but this is not confirmed by our evidence. Research should be conducted on the health outcomes and people-centeredness of different types of integration models to assess this common assumption.

Although it is commonly argued that integration of TCIM into health systems will assist countries in achieving SDG3 ("Ensure healthy lives and promote well-being for all at all ages"), there is a paucity of evidence on which models of integration have a positive impact on achievement of this SDG. The conceptual framework of models of integration of TCIM into health systems constitutes a first step in this direction. As outlined in the WHO Health System Performance

Framework for Policy Analysis (29), the next steps should be practical and conceptual linkage of these integration models with key health system functions and outcomes in order to assess the models. Research should also be conducted to assist countries in developing strategies to evaluate, monitor and improve integration of TCIM into their health systems. Assessment is required of how different models and levels of integration impact specific health system functions and ultimately attainment of specific health system goals and outcomes.

6 Conclusion

There is no ideal model of integration. Each country's unique context will determine the form of integration that is both desirable and practical. The theoretical framework described here enables consistent language and categorization of different approaches to integration that will facilitate sharing of best practices, and research to build the evidence-base regarding which models may be most effective in supporting the SDGs in specific contexts. Regardless of the model category, the evidence from the country site visits suggests that an important consideration in integrating TCIM into health systems is mutual respect and collaboration between providers of TCIM and biomedicine. The literature indicates that leadership support is necessary to support effective integration in all model categories, suggesting that some input of a centralized approach is necessary. It is not clear whether pluralistic or unification integration models are more effective and this is an important area for future research.

Countries are encouraged to use the conceptual framework to assess their integration model and to identify the building block(s) in which changes should be made, if changes to the current model are required. Use of the conceptual framework will also facilitate comparative and outcomes research to determine the models that are best suited to each context to enhance the health of populations. The next important step will be to develop, pilot-test and validate a tool for operationalization of the conceptual framework for categorizing models of integration.

References

1. Biomedicine. In: NCI Dictionary of Cancer Medicine (online). Bethesda (MD): National Cancer Institute; 2024. <https://www.cancer.gov/publications/dictionaries/cancer-terms/def/biomedicine>.
2. Quirke V, Gaudillière JP. The era of biomedicine: science, medicine, and public health in Britain and France after the Second World War. *Med Hist*. 2008;52(4):441–52. <https://doi.org/10.1017/s002572730000017x>.
3. WHO traditional medicine strategy: 2014–2023. Geneva: World Health Organization; 2013. <https://www.who.int/publications/i/item/9789241506096>.
4. World Health Organization, United Nations Children’s Fund. Operational framework for primary health care: transforming vision into action. Geneva: World Health Organization; 2020. <https://www.who.int/publications/i/item/9789240017832>. Licence: CC BY-NC-SA 3.0 IGO.
5. A glossary of technical terms on the economics and finance of health services. Copenhagen: World Health Organization Regional Office for Europe; 1998. <https://iris.who.int/items/89cab4fd-da13-478d-8e9e-ecaf28529596>.
6. The world health report 2000. Health systems: improving performance. Geneva: World Health Organization; 2000. <https://www.who.int/publications/i/item/924156198X>.
7. Global traditional medicine strategy 2025–2034. Geneva: World Health Organization; 2025. <https://www.who.int/publications/i/item/9789240113176>.
8. WHO global report on traditional and complementary medicine 2019. Geneva: World Health Organization; 2019. <https://www.who.int/publications/i/item/978924151536>.
9. World Health Assembly, 69 (2016) Sixty-ninth World Health Assembly, Geneva, 23–28 May 2016: resolutions and decisions, annexes. Geneva: World Health Organization; 2016. <https://iris.who.int/handle/10665/259134>.
10. Resolution adopted by the General Assembly on 5 October 2023. 78/4. Political declaration of the high-level meeting on universal health coverage. New York: United Nations; 2023. <https://documents.un.org/doc/undoc/gen/n23/306/84/pdf/n2330684.pdf>
11. WHO traditional medicine strategy 2002–2005. Geneva: World Health Organization; 2002 <https://www.who.int/publications/i/item/WHO-EDM-TRM-2002.1>.
12. Goals. 3. Ensure healthy lives and promote well-being for all at all ages. New York: United Nations, Department of Economic and Social Affairs; 2015. <https://sdgs.un.org/goals/goal3>.
13. Waddington C, Egger D. Integrated health services – what and why? (Technical brief No. 1). Geneva: World Health Organization; 2008. https://terrance.who.int/mediacentre/data/sage/SAGE_Docs_Ppt_Apr2014/10_session_child_health_services/Apr2014_session10_integrated_health_services.pdf.
14. United Nations Declaration on the Rights of Indigenous Peoples. Resolution adopted by the General Assembly on 13 September 2007. New York: United Nations; 2007. https://www.un.org/development/desa/indigenouspeoples/wp-content/uploads/sites/19/2018/11/UNDRIP_E_web.pdf.
15. C169 – Indigenous and Tribal Peoples Convention, 1989 (No. 169). Geneva: International Labour Organization; 1989. https://normlex.ilo.org/dyn/normlex/en/f?p=NORMLEXPUB:12100:0::NO::P12100_ILO_CODE:C169.
16. Convention on Biological Diversity. Text and annexes. Montreal: Secretariat of the Convention on Biological Diversity; 2011. <https://www.cbd.int/doc/legal/cbd-en.pdf>.
17. WIPO Treaty on Intellectual Property, Genetic Resources and Associated Traditional Knowledge, 2024. Geneva: World Intellectual Property Organization; 2024. https://www.wipo.int/edocs/mdocs/tk/en/gratk_dc/gratk_dc_7.pdf.

18. Canaway R. Integration of traditional and complementary medicine in South-East Asia: public health, safety and management. A report for WHO-SEARO. New Delhi: World Health Organization Regional Office for South-East Asia; 2015. https://www.researchgate.net/publication/315786376_Integration_of_traditional_and_complementary_medicine_in_South-East_Asia_Public_health_safety_and_management_A_report_for_WHO-SEARO.
19. Park YL, Canaway R. Integrating traditional and complementary medicine with national healthcare systems for universal health coverage in Asia and the Western Pacific. *Health Syst, Reform*. 2018;5(2). <https://doi.org/10.1080/23288604.2018.1539058>.
20. Kailin D. Initial strategies. In: Faass NJ, editor. Integrating complementary medicine into health systems. Gaithersburg (FL): Aspen Publications; 2001:44–58. <https://www.jblearning.com/catalog/productdetails/9780834212169>.
21. Wiese M, Oster C, Pincombe J. Understanding the emerging relationship between complementary medicine and mainstream health care: a review of the literature. *Health*. 2020;14(3):326–42. <https://doi.org/10.1177/1363459309358594>.
22. Boon H, Verhoef M, O'Hara D, Findlay B, Majid N. Integrative healthcare: arriving at a working definition. *Altern Ther Health Med*. 2004;10(5):48–56. PMID: 15478786.
23. Coulter I. Integration and paradigm clash. In: Tovey P, Easthope G, Adams J, editors. Mainstreaming of complementary and alternative medicine. London: Routledge; 2005:103–22. <https://www.routledge.com/Mainstreaming-Complementary-and-Alternative-Medicine-Studies-in-Social-Context/Tovey-Easthope-Adams/p/book/9780415267007>.
24. Boon HS, Kachan N. Integrative medicine: A tale of two clinics. *BMC Complement Altern Med*. 2008;8:32. <https://doi.org/10.1186/1472-6882-8-32>.
25. Witt CM, Perard M, Berman B, Berman S, Birdsall TC, Defren H et al. Using the framework of corporate culture in “mergers” to support the development of a cultural basis for integrative medicine – Guidance for building an integrative medicine department or service. *Patient Preference Adherence*. 2015;9:113–20. <https://doi.org/10.2147/PPA.S66778>.
26. Nguyen V, Leung J, Lewanczuk R, Vohra S, Amrhein C. Exploring traditional Chinese medicine in Alberta: Challenges and opportunities. *Trad Med Modern Med*. 2018;1(3):181–92. <https://doi.org/10.1142/S2575900018200033>.
27. Monitoring the building blocks of health systems: a handbook of indicators and their measurement strategies. Geneva: World Health Organization; 2010. <https://iris.who.int/items/ecbd9a5a-538f-43a7-8050-d18073d51bd4>.
28. Everybody's business: strengthening health systems to improve health outcomes: WHO's framework for action. Geneva: World Health Organization; 2007. <https://iris.who.int/handle/10665/43918>.
29. World Health Organization, European Observatory on Health Systems and Policies, Papanicolas I, Rajan D, Karanikolos M et al. Health system performance assessment: a framework for policy analysis. Geneva: World Health Organization; 2022. <https://iris.who.int/handle/10665/352686>. License: CC BY-NC-SA 3.0 IGO.

Bibliography

Bodeker G, Ong CK, Grundy C, Burford G, Shein K et al. WHO global atlas of traditional, complementary and alternative medicine. Kobe: WHO Centre for Health Development; 2025. <https://iris.who.int/handle/10665/43108>.

Coulter ID, Willis EM. The rise and rise of complementary and alternative medicine: a sociological perspective. *Med J Aust*. 2004;180(11):587–9. <https://doi.org/10.5694/j.1326-5377.2004.tb06099.x>.

Hoenders H, Appelo M, Jong J. Integrative medicine: a bridge between biomedicine and alternative medicine fitting the spirit of the age. *Sociol Mind*. 2012;2:441–6. <https://doi.org/10.4236/sm.2012.24057>.

OED Online. Oxford: Oxford University Press; 2021. www.oed.com/view/Entry/97341.

Papaicolsa I, Rajan D, Karanikolos M, Soucat A, Figueras J, editors. Health system performance assessment: a framework for policy analysis. Geneva: World Health Organization; 2022. <https://iris.who.int/handle/10665/352686>. License: CC BY-NC-SA 3.0 IGO.

Roberts JL, World Health Organization. Terminology: a glossary of technical terms on the economics and finance of health services. Copenhagen: World Health Organization Regional office for Europe; 1998:69. <https://apps.who.int/iris/handle/10665/108335>.

Tataryn DJ. Paradigms of health and disease: a framework for classifying and understanding complementary and alternative medicine. *J Altern Complement Med*. 2002;8(6):877–92. <http://doi.org/10.1089/10755530260511874>.

World Health Organization, United Nations Children’s Fund. Primary health care: transforming vision into action – operational framework. Geneva: World Health Organization; 2020 <https://iris.who.int/handle/10665/337641>. License: CC BY-NC-SA 3.0 IGO.

Annex 1. WHO mandates and background documents on integration of TCIM

WHA56.31 (2003). The Fifty-sixth World Health Assembly (1) Urges Member States in accordance with established national legislation and mechanisms:

where appropriate, to formulate and implement national policies and regulations on traditional and complementary and alternative medicine in support of the proper use of traditional medicine, and its integration into national health-care systems, depending on the circumstances in their countries;

WHA62.13 (2009). The Sixty-second World Health Assembly (2) requests the Director-General:

to continue providing policy guidance to countries on how to integrate traditional medicine into health systems, especially to promote, where appropriate, the use of traditional/indigenous medicine for primary health care, including disease prevention and health promotion, in line with evidence of safety, efficacy and quality taking into account the traditions and customs of indigenous peoples and communities;

WHA67.18 (2014): The Sixty-seventh World Health Assembly (3) requests the Director-General:

to continue to provide policy guidance to Member States on how to integrate traditional and complementary medicine services within their national and/or subnational health care system(s), as well as the technical guidance that would ensure the safety, quality and effectiveness of such traditional and complementary medicine services with emphasis on quality assurance;

WHA69.24 (2016): The Sixty-ninth World Health Assembly (4) urges Member States:

to integrate, where appropriate, traditional and complementary medicine into health services, based on national context and knowledge-based policies, while assuring the safety, quality and effectiveness of health services and taking into account a holistic approach to health;

WHA72.4 (2019): The Seventy-second World Health Assembly (5) urges Member States:

to consider integrating, as appropriate, safe and evidence-based traditional and complementary medicine services within national and/or subnational health systems, particularly at the level of primary health care, according to national context and priorities;

WHA76.16 (2023): The Seventy-sixth World Health Assembly (6) urges Member States:

to explore ways to integrate, as appropriate, safe and evidence-based traditional and complementary medicine services, within national and/or subnational health systems, particularly at the level of primary health care, and mental health and wellness services;

The Declaration of the International Conference on Primary Health Care; Alma-Ata, USSR, 1978 (7) states, inter alia, that:

The people have the right and duty to participate individually and collectively in the planning and implementation of their health care. and

Primary health care relies, at local and referral levels, on health workers, including physicians, nurses, midwives, auxiliaries and community workers as applicable, as well as traditional practitioners as needed, suitably trained socially and technically to work as a health team and to respond to the expressed health needs of the community.

WHO Traditional Medicine Strategy 2002–2005 (8) describes WHO's strategy for traditional medicine and its four main objectives of framing policy, enhancing safety, efficacy and quality, ensuring access and promoting rational use. The first objective, policy, includes:

integrating TCIM with national health care systems, as appropriate, by developing and implementing national TCIM policies and programmes.

The Beijing Declaration, adopted by the WHO Congress on Traditional Medicine, Beijing, China, 8 November 2008 (9)

Participants ... in accordance with national capacities, priorities, relevant legislation and circumstances, hereby make the following Declaration:

1. The knowledge of traditional medicine, treatments and practices should be respected, preserved, promoted and communicated widely and appropriately based on the circumstances in each country.
2. Governments have a responsibility for the health of their people and should formulate national policies, regulations and standards, as part of comprehensive national health systems to ensure appropriate, safe and effective use of traditional medicine.
3. Recognizing the progress of many governments to date in integrating traditional medicine into their national health systems, we call on those who have not yet done so to take action.
4. The communication between conventional and traditional medicine providers should be strengthened and appropriate training programmes be established for health professionals, medical students and relevant researchers.

The WHO traditional medicine strategy: 2014–2023 (10) was developed in response to resolution WHA62.13 (2009) on traditional medicine, which requests the Director-General, inter alia, to update the WHO traditional medicine strategy: 2002–2005, based on countries' progress and current new challenges in the field of traditional medicine.

The strategy lays out three main strategic objectives, each of which contains strategic directions and specific actions to guide Member States, partners, stakeholders and the secretariat in positioning TCIM within countries' health systems and to help health-care leaders to develop solutions that contribute to a broader vision of improved health and patient autonomy.

Strategic objective number three is:

To promote universal health coverage by integrating traditional and complementary medicine services into health care service delivery and self-health care.

Declaration of Astana. Global Conference on Primary Health Care: Astana, Kazakhstan, 25 and 26 October 2018 (11) on primary health care acknowledges the need to include traditional medicine knowledge and technologies in the delivery of primary health care. Significant increases in the regulation of TCIM products, practices and practitioners have created better conditions for integrating TCIM services into health-care service delivery and self-health care.

Political declaration of the high-level meeting on universal health coverage, adopted by the United Nations General Assembly, 5 October 2023 (12)

We commit ... to achieve the health-related Sustainable Development Goals and targets through the following actions:

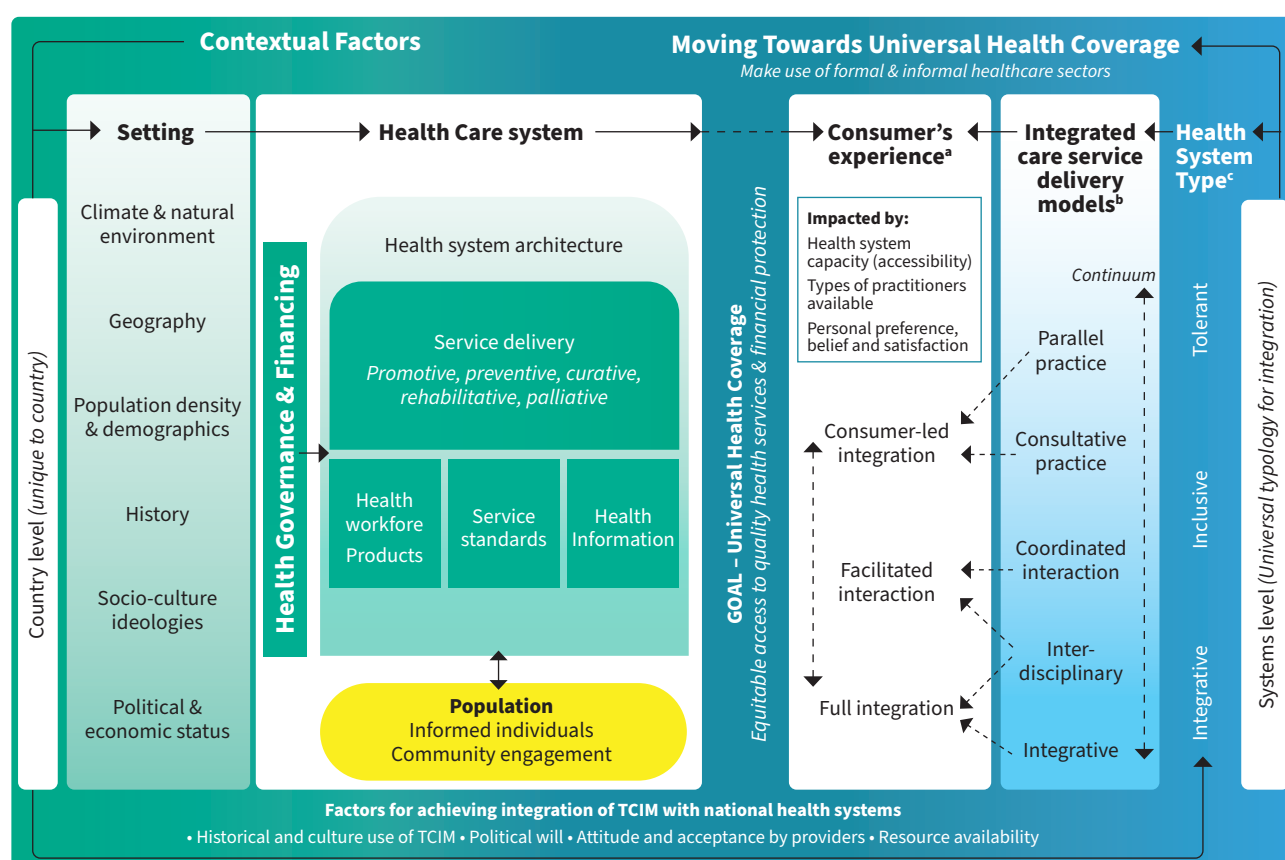
Continue to explore ways to integrate, as appropriate, safe and evidence-based traditional and complementary medicine services within national and local health systems, particularly at the level of primary health care, according to national context and priorities, while ensuring safety and quality of care, and in this regard recognize the important role and capacities of the World Health Organization and other relevant actors to support Member States with relevant evidence-based guidance.

References

1. Resolution WHA56.31. Traditional medicine. In: Fifty-sixth World Health Assembly, Geneva, 19–28 May 2003. Resolutions and decisions, annexes. Geneva: World Health Organization; 2003:63 (WHA56/2003/REC/1; https://apps.who.int/gb/ebwha/pdf_files/wha56/ea56r31.pdf).
2. Resolution WHA62.13. Traditional medicine. In: Sixty-second World Health Assembly, Geneva, 18–22 May 2009. Resolutions and decisions, annexes. Geneva: World Health Organization; 2009:19 (WHA62/2009/REC/1; https://apps.who.int/gb/ebwha/pdf_files/a62/a62_r13-en.pdf).
3. Resolution WHA67.18. Traditional medicine. In: Sixty-seventh World Health Assembly, Geneva, 19–24 May 2014. Resolutions and decisions, annexes. Geneva: World Health Organization; 2014:35 (WHA67/2014/REC/1; https://apps.who.int/gb/ebwha/pdf_files/wha67-rec1/a67_2014_rec1-en.pdf).
4. Resolution WHA69.24. Strengthening integrated, people-centred health services. In: Sixty-ninth World Health Assembly, Geneva, 23–28 May 2016. Resolutions and decisions, annexes. Geneva: World Health Organization; 2016:56 (WHA69/2016/REC/1; https://apps.who.int/gb/ebwha/pdf_files/WHA69-REC1/A69_2016_REC1-en.pdf).
5. Resolution WHA72.4. Preparation for the high-level meeting of the United Nations General Assembly on universal health coverage. In: Seventy-second World Health Assembly, Geneva, 20–28 May 2019. Resolutions and decisions, annexes. Geneva: World Health Organization; 2019:11 (WHA72/2019/REC/1; https://apps.who.int/gb/ebwha/pdf_files/WHA72-REC1/A72_2019_REC1-en.pdf).
6. Resolution WHA76.16. The health of Indigenous Peoples. In: Seventy-sixth World Health Assembly, Geneva, 21–30 May 2023. Resolutions and decisions, annexes. Geneva: World Health Organization; 2023:43 (WHA76/2023/REC/1; https://apps.who.int/gb/ebwha/pdf_files/WHA76-REC1/A76_REC1_Interactive_en.pdf).
7. Primary health care; report of the International Conference on Primary Health Care; Alma-Ata, USSR, 6–12 September 1978. Geneva: World Health Organization; 1978. <http://apps.who.int/iris/bitstream/10665/39228/1/9241800011.pdf>.
8. WHO traditional medicine strategy 2002–2005. Geneva: World Health Organization; 2002. <https://www.who.int/publications/i/item/WHO-EDM-TRM-2002.1>.
9. WHO congress on traditional medicine and the Beijing Declaration. Geneva: World Health Organization; 2009. <https://iris.who.int/handle/10665/74355>.
10. WHO traditional medicine strategy: 2014–2023. Geneva: World Health Organization; 2013. <https://apps.who.int/iris/handle/10665/92455>.
11. Declaration of Astana. Global Conference on Primary Health Care: Astana, Kazakhstan, 25 and 26 October 2018. Geneva: World Health Organization; 2018. <https://www.who.int/publications/i/item/WHO-HIS-SDS-2018.61>.
12. Resolution adopted by the General Assembly on 5 October 2023. 78/4. Political declaration of the high-level meeting on universal health coverage. New York: United Nations; 2023. <https://documents.un.org/doc/undoc/gen/n23/306/84/pdf/n2330684.pdf>.

Annex 2. Conceptual framework for integration of TCIM into health systems

Fig. A2.1. Conceptual framework for integration of TCIM into health systems



Sources: ^aBoon et al. (1); ^bLin et al. (2); ^cWHO (3)

See also Park & Canaway (4).

References

1. Boon H, Verhoef M, O'Hara D, Findlay B, Majid N. Integrative healthcare: arriving at a working definition. *Altern Ther Health Med*. 2004;10(5):48–56. PMID:15478786.
2. Lin V, Canaway R, Carter B. Interface, interaction and integration: how people with chronic disease in Australia manage CAM and conventional medical services. *Health Expect*. 2015;18(6):2651–65. <https://doi.org/10.1111/hex.12239>.
3. WHO traditional medicine strategy 2002–2005. Geneva: World Health Organization, Programme on Traditional Medicine; 2002). <https://iris.who.int/handle/10665/67163>.
4. Park YL, Canaway R. Integrating traditional and complementary medicine with national healthcare systems for universal health coverage in Asia and the Western Pacific. *Health Syst Reforms* 2019;5(1):24–31. <https://doi.org/10.1080/23288604.2018.1539058>.

Annex 3. Integration assessment tool (IAT) for consultation

The tool described below, IAT, is being pilot-tested by WHO.

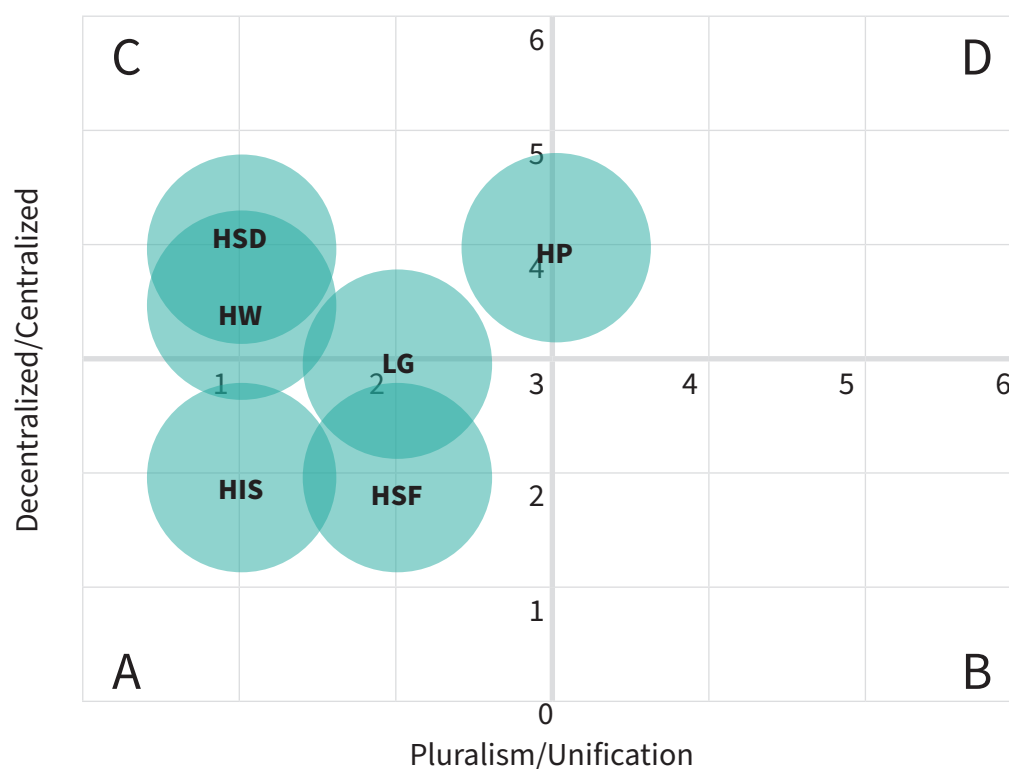
A3.1 Introduction

This tool provides guidance on criteria that can be used by countries to categorize their model of integration of TCIM into their health systems into one of four categories (A, people-led; B, practitioner-led; C, coordinated; and D, blended) in two dimensions (decentralized and centralized; pluralism and unification) with respect to the following six health system building blocks:

- leadership and governance
- health system financing
- health service delivery
- health workforce
- health information systems
- health products, vaccines and technologies

See Fig. A3.1.

Fig. A3.1. Theoretical example of using IAT in a country



LG, leadership and governance; HSF, health system financing; HSD, health service delivery; HW, health workforce; HIS, health information systems; HP, health products, vaccines and technologies

Fig. A3.1 shows the output of a theoretical assessment of a country in which integration of TCIM into the health system is primarily in categories A (people-led) and C (coordinated). In this case, the leadership and governance building block provides no clear indication which of the two categories is the goal of the health system. With health service delivery and the health workforce tending toward category C, a coordinated system would appear to be the most practical solution in this country. If a political decision is made to move more formally toward a coordinated system, the assessment indicates that changes will be required in health funding and in health information systems.

The tool can be used by countries to self-assess their health systems and/or by researchers and others to conduct comparative analyses of different models of integration. It could also be used to assess changes in the same model of integration over time.

The tool may be applied to a country as a whole or to a specific component of the health system, such as to assess integration of a specific group of practitioners or sites of care.

A3.2 Using the IAT

The tool is organized according to the six health system building blocks in column 1. Users may assess a single building block or use the tool to assess the entire health system (all six building blocks).

The tool is designed to yield scores for two dimensions – decentralized versus centralized; pluralism versus unification – for each of the health system building block. The two scores can be used as coordinates to plot each building block in the conceptual framework grid (see Table A3.1) to determine the category (or quadrant on the grid) into each one falls. When all six building blocks are plotted, they provide a picture of the country's overall model of integration of TCIM into its health systems.

Column 2 of the tool includes the data indicators, which are largely derived from data provided periodically by countries for WHO global surveys of TCIM. Column 2 for each building block is divided into two sections: one for each of the key dimensions of the framework (decentralized versus centralized; pluralism versus unification). Column 2 also contains indicators (questions or data inputs) relevant to assessing each building block for each of the two dimensions. The indicators associated with each dimension of the relevant building block are used to assign a score (from 1 to 5) for each dimension of each building block.

Scores for each relevant indicator can be recorded in column 3. A score of 3 is recorded for an indicator that is “neutral” (i.e. does not suggest a particular valence with respect to the given dimension). Once all the relevant indicators have been scored, an average dimension score can be determined by calculating the simple mean of all the indicator scores for the dimension.

This exercise is repeated for each building block. The result is 12 average dimension scores: two scores for each of the six building blocks. The average dimension scores can then be used as coordinates to locate each building block on a four-quadrant grid, providing a visual summary of a given country's integration model (Fig. A3.1).

It is expected that most countries will not have models that fall neatly into a single integration category; however, many will have a dominant category into which many or most of the health system building block scores fall, with some influence from other categories.

The IAT provides initial guidance based on current evidence that will be updated and refined during its use in countries with different TCIM integration models. Implementation and assessment will indicate the most important indicators for each dimension of each health system building block to create a final set of recommended indicators. Validation of the tool is beyond the scope of this guidance document. The tool is currently being pilot-tested by WHO.

Three country examples assessed with the tool in a proof-of-concept exercise are provided below.

Table A3.1. IAT for identifying the model of integration of TCIM into health systems

Health system building block	Indicators					
Leadership and governance	Dimension: Decentralized versus centralized					
	Scoring System		1	2	3	4 5
	1. No national TCIM policy; generally, limited regulation or oversight of TCIM; TCIM practices are tolerated.					
	2. No national TCIM policy; little regulation or oversight of TCIM; TCIM practices are legal.					
	3. Neutral or not available (NA)					
	4. National TCIM policy; some regulation or oversight of TCIM					
	5. National TCIM policy; TCIM generally regulated					
	Indicators					
	I. Is there a national policy, law or regulation on TCIM? Consider:					
	– the extent and nature of government policy or legislation that mandates or supports integration of TCIM into the health system					
	II. What are the extent and nature of government regulation of TCIM health care practitioners and practices? Consider:					
	– the scope of practice and protected titles (if any) of TCIM practitioners;					
	– whether there is licencing, registration or certification; and					
	– where TCIM practitioners are unregulated, consider their legal status (e.g. illegal? illegal but tolerated? legal?)					
	III. Is there a national office for TCIM?					
	Average dimension score:					
	Dimension: pluralism versus unification					
	Scoring System		1	2	3	4 5
	1. TCIM is recognized as fully distinct from biomedicine in any regulation, policy or framework.					
	2. TCIM is usually recognized as distinct from biomedicine in any regulation, policy or framework.					
	3. Neutral or NA					
	4. TCIM is sometimes conceptualized as blended with biomedicine in any regulation or policy.					
	5. TCIM is usually or fully conceptualized as blended with biomedicine in any regulation or policy.					
	Indicators					
	IV. To what extent are TCIM practices recognized as distinct from biomedicine practices in any government policy or regulation? Consider:					
	To what extent does any government policy or legislation mandate or support integration described as pluralism or unification?					
	V. To what extent does any government regulation of health-care practitioners recognize TCIM practitioners as separate and distinct from biomedicine practitioners, or are TCIM practitioners and biomedicine practitioners regulated in a single, blended system?					
	VI. To what extent does any government regulation recognize TCIM health products as separate and distinct, or are they regulated in a single blended system with biomedicine products?					
	VII. To what extent does any government regulation recognize TCIM practitioner education as separate and distinct, or is TCIM education regulated in a single blended system with biomedicine education?					
	Average dimension score:					

Health system financing	Dimension: Decentralized versus centralized				
	Scoring System				
	1	2	3	4	5
	1. There is minimal core funding for TCIM.				
	2. There is minimal core funding for TCIM; supplementary or discretionary funding is available.				
	3. Neutral or NA				
	4. There is moderate core funding for TCIM.				
	5. TCIM is generally funded as part of the health-care package.				
	Indicators				
	I. To what extent is there funding for TCIM services, treatments and products? Consider:				
	– public sector TCIM and biomedicine practitioners' salaries and benefits reimbursement; and				
	– the extent to which TCIM is included in any publicly funded essential health services package (provided in public health facilities).				
	II. What is the nature and extent of coverage of TCIM practices by health insurance?				
	III. To what extent are TCIM services, treatments and products paid for out of pocket by individuals?				
Average dimension score:					
Dimension: Pluralism versus unification					
Scoring System					
1	2	3	4	5	
1. Any TCIM funding is distinct.					
2. Any TCIM funding is usually distinct.					
3. Neutral or NA					
4. Any TCIM funding is sometimes blended with biomedicine funding.					
5. Any TCIM funding is usually or fully blended with biomedicine funding.					
Indicators					
IV. To what extent is any funding for TCIM distinct and separate or blended with funding for biomedicine practitioner services, treatments and products?					
V. To what extent is any insurance coverage for TCIM practices distinct and separate or blended with funding for biomedicine practices?					
Average dimension score:					

Health service delivery	Dimension: Decentralized versus centralized				
	Scoring System	1	2	3	4 5
	1. TCIM health services are not delivered as part of the government-supported health system.				
	2. TCIM health services are generally not delivered as part of the government-supported health system.				
	3. Neutral or NA				
	4. TCIM health services are generally delivered as part of the government-supported health system.				
	5. TCIM health services are delivered as part of the government-supported health system.				
	Indicators				
	I. How are TCIM health services delivered? Consider:				
	– The extent to which there is government oversight or support for the delivery of TCIM health services within the health system				
	– Number and type of TCIM practitioners employed by the public sector health system				
	II. How accessible are TCIM services to individuals throughout the country? Consider:				
	– The proportion of health facilities that offer TCIM				
	III. Consider the extent to which any quality assurance activities include TCIM health facilities.				
	Average dimension score:				
	Dimension: Pluralism versus unification				
	Scoring System	1	2	3	4 5
	1. TCIM health services are distinct.				
	2. TCIM health services are usually distinct.				
	3. Neutral or NA				
	4. TCIM health services are sometimes blended with biomedicine health services.				
	5. TCIM health-care services are usually or fully blended with biomedicine services.				
	Indicators				
	IV. How are TCIM health services delivered (are they distinct services or blended with biomedicine health services)? Consider:				
	– The extent to which people experience distinct or blended care from biomedicine and TCIM practitioners.				
	V. To what extent do publicly funded health delivery facilities include TCIM?				
	Average dimension score:				

Health workforce	Dimension: Decentralized versus centralized				
	Scoring System	1	2	3	4 5
	1. TCIM practice and education are not regulated.				
	2. TCIM practice and education are generally not regulated, and biomedicine practitioners may include some TCIM practices.				
	3. Neutral or NA				
	4. There is some oversight or regulation of TCIM practice and education.				
	5. TCIM practice and education are overseen and regulated by the government.				
	Indicators				
	I. To what extent do people have access to health services in the national and local health systems provided by a range of health-care providers, including TCIM practitioners? Consider:				
	– What kinds and how many TCIM practitioners are practising?				
	II. How much government oversight (e.g. regulation) is there of the practices of TCIM health-care practitioners? Consider:				
	– the number and type of TCIM practitioners licensed or registered in the country				
	– Is there a licensing examination?				
	– Are there continuing education requirements?				
	III. How much government oversight is there of the education and professional development of TCIM health-care practitioners? Consider:				
	– the number and highest degree offered by TCIM educational institutions				
	– Is TCIM education provided at university level?				
	– Is there a government-supported professional development programme for TCIM providers?				
	Average dimension score:				
	Dimension: Pluralism versus unification				
	Scoring system	1	2	3	4 5
	1. TCIM and biomedicine practitioners have distinct and independent education and practices.				
	2. TCIM and biomedicine practitioners have distinct and independent education and practices but recognize the value of learning about each others' practices.				
	3. Neutral or NA				
	4. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap to some extent.				
	5. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap significantly.				
	Indicators				
	IV. How much overlap is there in the education of TCIM and biomedicine providers? Consider:				
	– How much training do TC&M practitioners receive in biomedicine?				
	– How much training do biomedicine practitioners receive in TCIM?				
	V. How much overlap is there in the scope of practice of TCIM and biomedicine providers? Consider:				
	– Can TCIM practitioners prescribe or practise biomedicine?				
	– Can biomedicine practitioners prescribe or practise TCIM?				
	– If there are practice guidelines, are there separate TCIM practice guidelines, or are those for TCIM included in general clinical practice guidelines?				
	Average dimension score:				

Health information systems	Dimension: Decentralized versus centralized									
	Scoring System					1	2	3	4	5
	1. TCIM and biomedicine practitioners sometimes voluntarily and informally share information.									
	2. TCIM and biomedicine practitioners regularly voluntarily share information.									
	3. Neutral or NA									
	4. TCIM and biomedicine practitioners are sometimes required to share information.									
	5. TCIM and biomedicine practitioners are usually required to share information.									
	Indicators									
	I. To what extent are TCIM biomedicine practitioners required to share information with each other? Consider:									
	– What systems or technology exist to support information-sharing?									
	– Who has access to patient record systems and relevant patient data?									
	– Who controls access to patient record systems and relevant patient data?									
Average dimension score:										
	Dimension: Pluralism versus unification									
	Scoring System					1	2	3	4	5
	1. TCIM and biomedicine practitioners have distinct, independent record-keeping practices.									
	2. TCIM and biomedicine practitioners have distinct record-keeping practices but recognize the value of coordinating information with others.									
	3. Neutral or NA									
	4. The record-keeping and information systems of TCIM and biomedicine practitioners have some shared requirements and facilitate some information-sharing.									
	5. The record-keeping and information systems of TCIM and biomedicine practitioners have many shared requirements and facilitate sharing of information.									
	Indicators									
	II. How much overlap is there in the billing, charting, referral, health data and information systems of TCIM and biomedicine providers? Consider:									
	– What, if any, structures or processes are in place to support communication and collaboration between TCIM and biomedicine practitioners?									
	– Who has access to patient record systems and relevant patient data?									
	– Is a standard format (e.g. the International Classification of Diseases) used to code health conditions, and does it (and if so, how) include TCIM practitioners and/or diagnoses?									
	Average dimension score:									

Health products, vaccines and technologies	Dimension: Decentralized versus centralized					
	Scoring System	1	2	3	4	5
	1. No regulation of TCIM products					
	2. Limited regulation of TCIM products					
	3. Neutral or NA					
	4. TCIM health products, vaccines and technologies are regulated to some extent by the government.					
	5. TCIM health products and medical products are regulated.					
	Indicators					
	I. To what extent are TCIM health products, vaccines and technologies regulated by the government?					
	II. To what extent are the quality, safety, efficacy and cost-effectiveness of TCIM health products and technologies assessed by the government? Consider:					
	– Are good manufacturing practices applied to TCIM products?					
	– Are there mechanisms to ensure compliance with good manufacturing practices ?					
	III. To what extent is the sale (who can sell and where) of TCIM products regulated? Consider:					
	– Are TCIM products sold in pharmacies, in other outlets, by licensed practitioners?					
	– Are there any restrictions on who can sell TCIM products or where?					
	IV. Does the national pharmacopoeia includes TCIM products such as herbal medicines?					
	V. Is there post-marketing surveillance or a pharmacovigilance system for TCIM products?					
	Average dimension score:					
	Dimension: Pluralism versus unification					
	Scoring System	1	2	3	4	5
	1. TCIM health products and technologies are distinctly and independently regulated, with distinct and independent routes of access.					
	2. TCIM health products and technologies are generally regulated distinctly and independently and are accessed by distinct and independent routes.					
	3. Neutral or NA					
	4. Regulation of and access to TCIM and biomedicine health products have shared requirements and pathways.					
	5. Regulation of and access to TCIM and biomedicine products and technologies overlap significantly with respect to shared requirements and pathways.					
	Indicators					
	VI. To what extent is there overlap in the regulation, quality control and adverse event reporting of TCIM health products and technologies? Consider:					
	– Are the standards for TCIM products the same as or different from those for biomedicine products?					
	VII. To what extent is there overlap in the way people access TCIM products and technologies? Consider:					
	– Are the access routes for TCIM products the same or different?					
	– If there is a national essential medicines list. Does it include TCIM products?					
	VIII. To what extent are any good manufacturing practices the same for TCIM products and biomedicines?					
	IX. To what extent does any national pharmacopoeia include both TCIM products and biomedicine products?					
	X. To what extent does any post-marketing surveillance or pharmacovigilance system include both biomedicine and TCIM products?					
	Average dimension score:					

Annex 4. Examples of application of the tool in three countries

Information for this document was contributed by countries in all six WHO regions. The examples below are based on data collected by the expert group and their experiences during the site visits.

This exercise was intended to provide “proof of concept”. It shows that the tool is helpful in identifying a country’s model of integration of TCIM into their health systems. The descriptions below are not intended as comprehensive assessments of the current models of integration in any of the countries. The data used for this example were collected by the expert group at a specific time and should not be assumed to be accurate representations of the current status.

The three countries were chosen for site visits because they are known to have different approaches to integration of TCIM into their health systems. The differences were confirmed during the site visits. Thus, the tool appears to have face validity because it clearly differentiates the three approaches taken in the three countries in ways consistent with the qualitative experiences of those who undertook the site visits.

The tool requires formal pilot-testing and assessment in order to identify and refine the indicators for each dimension of each building block. The instructions should be clarified to increase the ease of use, and the outputs of the tool should be validated by comparing them with each country’s descriptions of the integration model and goals. Overall, this first application of the tool suggests that it might be helpful for increasing consistency by comparing integration approaches among and within the same country over time.

Country A

Country A has one of the most comprehensive and complex TCIM systems in the world. The country is integrating biomedicine with at least six other systems of medicine, including some which are clearly rooted in the country’s traditional culture and others which have arrived from abroad.

Country A’s approach to integration is strongly centralized. As the systems of medicine that are being integrated are recognized as distinct, it is a primarily pluralistic system. As can be seen in Fig. A4.1, Country A’s approach can be categorized as primarily coordinated, in category C, with some of the health system building blocks trending towards blended, category D.

Country B

Country B is a deeply religious country, with a mix of many religions and associated traditional medicine especially in rural areas. There is strong political will from the Government of Country B for integration of TCIM into its national health systems.

Country B’s approach to integration is in transition, moving from a primarily decentralized to a more centralized system. Their pluralistic system recognizes the distinct natures of traditional medicine and biomedicine (Fig. A4.1). The approach is partly decentralized and partly centralized and primarily pluralistic, with some health system building blocks in the people-led category A, and coordination of some building blocks, category C.

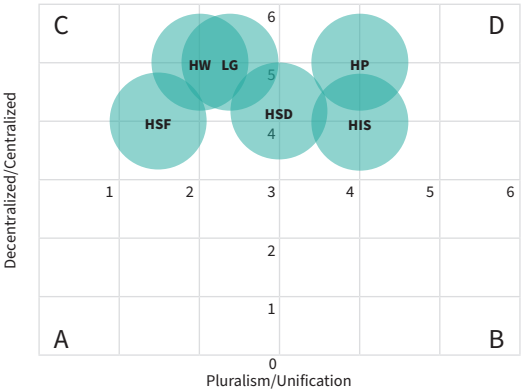
Country C

In Country C, traditional medicine is not regulated and is learnt by oral tradition. Complementary medicine approaches such as acupuncture, homeopathy and naturopathy have been legalized in Country C.

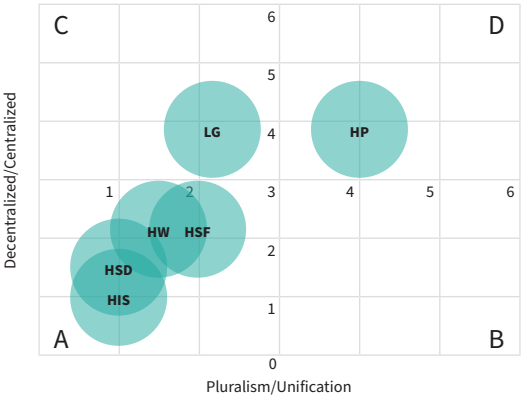
Country C’s approach to integration is a mix of centralized and decentralized and is generally pluralistic. As seen in Fig. A4.1, the health system building blocks are primarily in the people-led category, A, with Government commitment to movement towards coordinated category C.

Fig. A4.1. Use of the IAT to analyse integration of TCIM into the health systems of three example countries

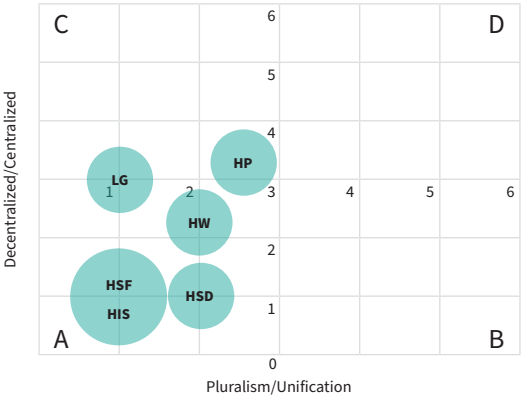
Country A



Country B



Country C

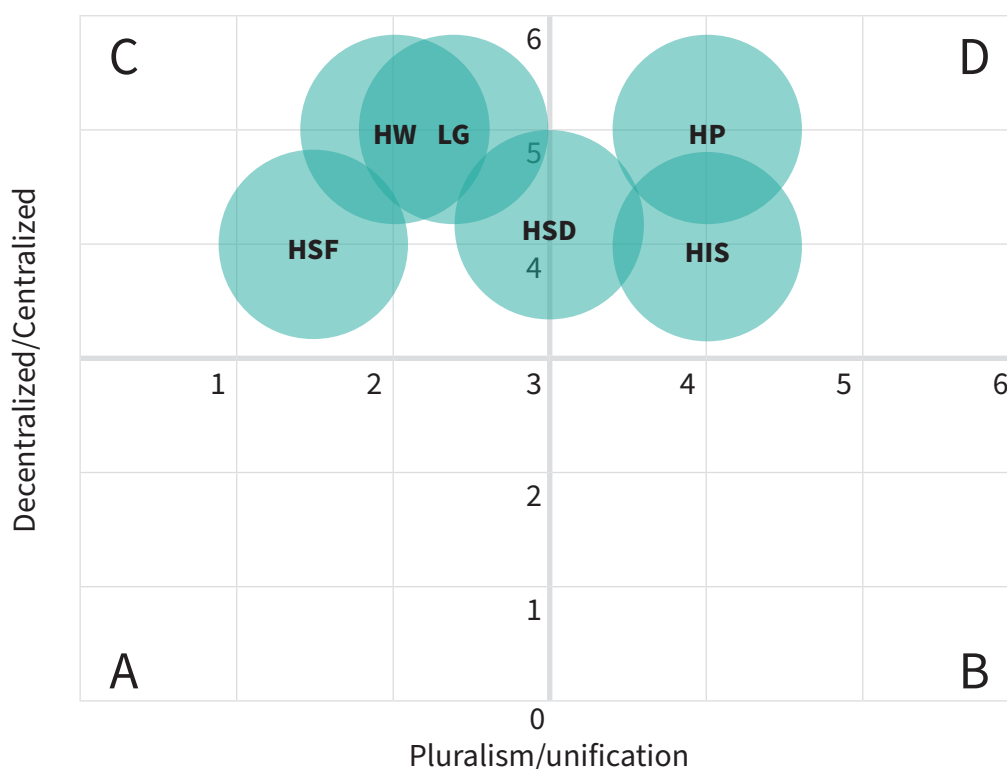


LG, leadership and governance; HSF, health system financing; HSD, health service delivery; HW, health workforce; HIS, health information systems; HP, health products, vaccines and technologies

A4.3 Country A

Country A has a rich, complex traditional medicine system that includes many TCIM systems in many different ethnic cultures. In addition to many established indigenous systems of medicine, several complementary systems have been adopted from abroad. Attempts are being made to integrate biomedicine with TCIM systems of care.

Fig. A4.2. Country A Summary



LG, leadership and governance; HSF, health system financing; HSD, health service delivery; HW, health workforce; HIS, health information systems; HP, health products, vaccines and technologies

Health system building block	Indicators					
Leadership and governance	Dimension: decentralized versus centralized					
	Scoring System		1	2	3	4 5
	1. No national TCIM policy; generally, limited regulation or oversight of TCIM; TCIM practices are tolerated.					
	2. No national TCIM policy; little regulation or oversight of TCIM; TCIM practices are legal.					
	3. Neutral or not available (NA)					
	4. National TCIM policy; some regulation or oversight of TCIM					
	5. National TCIM policy; TCIM generally regulated					
	Indicators					
	I. Is there a national policy, law or regulation on TCIM?					
	Answer: Yes.					
	II. What are the extent and nature of Government regulation of TCIM health-care practitioner and practices?					5
	Answer: Country A has a well-developed federal regulatory system at central and state levels for education, practice and products.					
	III. Is there a national office for TCIM?					5
	Answer: Yes.					
	Average dimension score: 5					5
	Dimension: Pluralism versus unification					
	Scoring System		1	2	3	4 5
	1. TCIM is recognized as fully distinct from biomedicine in any regulation, policy or framework.					
	2. TCIM is usually recognized as distinct from biomedicine in any regulation, policy or framework.					
	3. Neutral or NA					
	4. TCIM is sometimes conceptualized as blended with biomedicine in any regulation or policy					
	5. TCIM is usually or fully conceptualized as blended with biomedicine in any regulation or policy.					
	Indicators					
	IV. To what extent are TCIM practices recognized as distinct from biomedical practices in any government policy or regulation?				2	
	Answer: Pluralistic health care management is the key policy principle of Country A's national health plan. Patients have access to TCIM care providers. These systems also have government support in research and supervision to develop and enrich their contributions to meeting the national health goals and objectives through integrative practices.					
	V. To what extent does any government regulation of health-care practitioners recognize TCIM practitioners as separate and distinct from biomedicine practitioners, or are TCIM practitioners and biomedicine practitioners regulated in a single, blended system?				2	
	Answer: The national health plan of Country A advocates pluralism.					
	VI. To what extent does any government regulation recognize TCIM health products as separate and distinct, or are they regulated in a single blended system with conventional medical products?					4
	Answer: Conventional and traditional medicines are regulated under the same Act.					
	VII. To what extent does any government regulation recognize TCIM practitioner education as separate and distinct, or is TCIM education regulated in a single blended system with conventional medical education?				2	
	Answer: The standards of education, teaching and training, recognition of degrees, and practice of TCIM are regulated by several distinct Acts					
	Average dimension score: 2.5					

Health system financing	Dimension: Decentralized versus centralized				
	Scoring System				
	1	2	3	4	5
	1. There is minimal core funding for TCIM.				
	2. There is minimal core funding for TCIM; supplementary or discretionary				
	3. Neutral or NA				
	4. There is moderate core funding for TCIM.				
	5. TCIM is generally funded as part of the health-care package.				
	Indicators				
	I. To what extent is there funding for TCIM services, treatments and products?				
	Answer: In the Government health system, similar co-payment by the patient is required for allopathic, traditional and complementary systems of health care.				
	4				
	II. What are the nature and extent of coverage of TCIM practice by health insurance?				
	Answer: In the private sector, health insurance coverage determines the extent to which health care is covered. About 27 insurance companies offer more than 140 policies covering TCIM.				
	4				
III. To what extent are TCIM services, treatments and product paid for out of pocket by individuals?					
Answer: There is some out-of-pocket payment by patients					
4					
Average dimension score: 4					
Dimension: Pluralism versus unification					
Scoring System					
1	2	3	4	5	
1. Any TCIM funding is distinct.					
2. Any TCIM funding is usually distinct.					
3. Neutral or NA					
4. Any TCIM funding is sometimes blended with biomedicine funding.					
5. Any TCIM funding is usually or fully blended with biomedicine funding.					
Indicators					
IV. To what extent is any funding for TCIM distinct and separate or blended with funding for biomedicine practitioner services, treatments and products?					
Answer: The Government provides separate funding for the TCIM and conventional systems of medicine, which are governed by two separate Government ministries.					
1					
V. To what extent is any insurance coverage for TCIM practices distinct and separate or blended with funding for biomedicine practices?					
Answer: Under the regulations of the Insurance Regulatory and Development Authority, about 27 insurance companies offer more than 140 policy products covering TCIM.					
2					
Average dimension score: 1.5					

Health service delivery	Dimension: Decentralized versus centralized					
	Scoring System:		1	2	3	4 5
	1. TCIM health services are not delivered as part of the government-supported health system.					
	2. TCIM health services are generally not delivered as part of the government-supported health system.					
	3. Neutral or NA					
	4. TCIM health services are generally delivered as part of the government-supported health system.					
	5. TCIM health services are delivered as part of the government-supported health system.					
	Indicators					
	I. How are TCIM health services delivered? Answer: In integrated health centres					4
	II. How accessible are TCIM services to individuals throughout the country? Answer: The Government has established that health services and home care are delivered by traditional, complementary and biomedicine practitioners in collaboration.					5
	III. Consider the extent to which any quality assurance activities include TCIM health facilities. Answer: There is a national accreditation board for health-care organizations, including for those that provide TCIM. * Separate accreditation standards and requirements have been established for different systems of medicine, which are aligned with hospital standards, with a community focus. There is a national accreditation system for health-care providers, including TCIM providers.					4
	Average dimension score: 4.3					
	Dimension: Pluralism versus unification					
	Scoring System		1	2	3	4 5
	1. TCIM health services are distinct.					
	2. TCIM health services are usually distinct.					
	3. Neutral or NA					
	4. TCIM health services are sometimes blended with biomedical health services.					
	5. TCIM health-care services are usually or fully blended with biomedical services.					
	Indicators					
	IV. How are TCIM health services delivered (are they distinct services or blended with biomedicine health services)? Answer: In primary health care, integration of practitioners is made possible through bridging courses, in which basic training in western medicine and emergency skills are taught to a TCIM practitioner. At secondary and tertiary care levels, there is a strong preference for multidisciplinary approaches, with separate practitioners working together. There are national accreditation quality standards for each TCIM system. Each system has separate accreditation standards according to their medicines and requirements.					2
	V. To what extent do publicly funded health facilities include TCIM? Answer: The Government of Country A has adopted a strategy of co-location of TCIM facilities within primary health care centres, community health centres and district hospitals, thus giving patients the choice of different systems of medicine under a single roof.					4
	Average dimension score: 3					

Health workforce	Dimension: Decentralized versus centralized					
	Scoring System	1	2	3	4	5
	1. TCIM practice and education are not regulated.					
	2. TCIM practice and education are generally not regulated, and biomedical practitioners may include some TCIM practices.					
	3. Neutral or NA					
	4. There is some oversight or regulation of TCIM practice and education.					
	5. TCIM practice and education are overseen and regulated by the government.					
	Indicators					
	I. To what extent do people have access to health services and care in the national and local health systems provided by a range of health-care providers, including TCIM? Answer: The government of Country A has adopted a strategy of co-location of TCIM facilities in primary health care centres, community health centres and district hospitals, giving patients the choice of different systems of medicine under a single roof.					5
	II. How much government oversight (e.g. regulations) is there of the practices of TCIM health-care practitioners? Answer: The health authorities regulate providers of traditional, complementary and biomedicine in equivalent systems of regulation. There is some variation among states in the scope of TCIM practice. The government provides assessment and accreditation for TCIM educational institutions.					5
	III. How much government oversight is there of the education and professional development of TCIM health-care practitioners? Answer: To practise any form of medicine, a practitioner must be registered (unregistered practice is illegal); some traditional practitioners such as faith healers remain outside the system.					5
	Average dimension score: 5					
	Dimension: Pluralism versus unification					
	Scoring System	1	2	3	4	5
	1. TCIM and biomedicine practitioners have distinct, independent education and practices.					
	2. TCIM and biomedicine practitioners have distinct, independent education and practices but recognize the value of understanding each others' practices.					
	3. Neutral or NA					
	4. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap to some extent.					
	5. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap significantly.					
	Indicators					
	IV. How much overlap is there in the education of TCIM and biomedicine providers? Answer: In biomedicine professional education, there is generally no theoretical or practical training in TCIM. This is changing in some university programmes. In TCIM professional education, some theoretical and/or practical training is given in biomedicine. A national eligibility entrance test is common for entrance into graduate courses for both conventional and some TCIM systems. Both conventional and some TCIM medicines are regulated under the same Act.					2
	V. How much overlap is there in the scope of practice of TCIM and biomedicine providers? Answer: They have the same job opportunities in public health, but their practices are distinct.					2
	Average dimension score: 2					

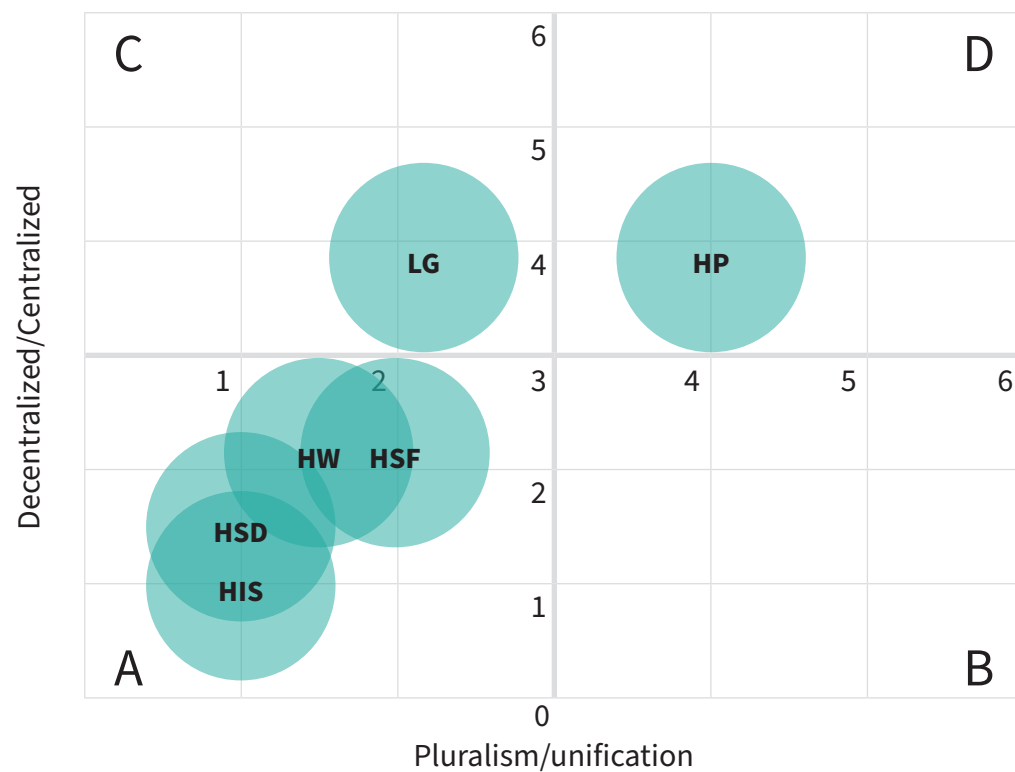
Health information systems	Dimension: Decentralized versus centralized					
	Scoring System		1	2	3	4 5
	1. TCIM and biomedicine practitioners sometimes voluntarily and informally share information.					
	2. TCIM and biomedicine practitioners regularly voluntarily share information.					
	3. Neutral or NA					
	4. TCIM and biomedicine practitioners are sometimes required to share information.					
	5. TCIM and biomedicine practitioners are usually required to share information.					
	Indicators					
	I. To what extent are TCIM biomedicine practitioners required to share information with each other?					
	Answer: The health management information system is a government web-based monitoring information system for monitoring health programmes and providing input for policy formulation and appropriate programme interventions.					4
	Data from some mobile health facilities are also included in the Government information system.					
	There is a separate hospital management system for TCIM electronic health records, with different modules to retain the uniqueness of different TCIM systems.					
	Average dimension score: 4					
	Dimension: Pluralism versus unification					
	Scoring System		1	2	3	4 5
	1. TCIM and biomedicine practitioners have distinct, independent record-keeping practices.					
	2. TCIM and biomedicine practitioners have distinct record-keeping practices but recognize the value of coordinating information-sharing.					
	3. Neutral or NA					
	4. The record-keeping and information systems of TCIM and biomedicine practitioners share some requirements and facilitate some information-sharing.					
	5. The record-keeping and information systems of TCIM and biomedicine practitioners share many requirements and facilitate sharing of information.					
	Indicators					
	II. How much overlap is there in billing, charting, referral data and information systems of TCIM and biomedicine providers?					4
	Answer: See answer above.					
	Average dimension score: 4					
Health products, vaccines and technologies	Dimension: Decentralized vs centralized					
	Scoring System		1	2	3	4 5
	1. No regulation of TCIM products					
	2. Limited regulation of TCIM products					
	3. Neutral or NA					
	4. TCIM health products, vaccines and technologies are somewhat regulated by the government.					
	5. TCIM health products and medical products are regulated.					
	Indicators					
	I. To what extent are TCIM health products, vaccines and technologies regulated by the government?					
	Answer: The health authorities regulate products of traditional, complementary and biomedicine through equivalent systems of regulation.					5

II. To what extent are the quality, safety, efficacy and cost-effectiveness of TCIM health products, vaccines and technologies assessed by the government?					5
Answer: The health authorities regulate products of traditional, complementary and biomedicine through equivalent systems of regulation.					
III. To what extent is the sale (who can sell and where) of TCIM products regulated?					5
Answer: The health authorities regulate products of traditional, complementary and biomedicine through equivalent systems of regulation.					
IV. Does the national pharmacopoeia include TCIM products such as herbal medicines?					5
Answer: The national monographs of TCIM on single herbs and formulations are included in the national pharmacopoeias.					
V. Is there post-marketing surveillance or a pharmacovigilance system for TCIM products?					5
Answer: yes there is a national pharmacovigilance program.					
Average dimension score: 5					
Dimension: Pluralism vs unification					
Scoring System	1	2	3	4	5
1. TCIM health products and technologies are distinctly and independently regulated, with distinct and independent routes of access.					
2. TCIM health products and technologies are generally regulated distinctly and independently and are accessed by distinct and independent routes.					
3. Neutral or NA					
4. Regulation of and access to TCIM and biomedicine health products have shared requirements and pathways.					
5. Regulation of and access to TCIM and biomedicine products and technologies overlap significantly with respect to shared requirements and pathways.					
Indicators					
VI. To what extent is there overlap in the regulation, quality control and adverse event reporting of TCIM health products, vaccines and technologies?					5
Answer: The standards for the safety, efficacy and quality of herbal medicines are the same as those for biomedicine.					
VII. To what extent is overlap in the way people access TCIM health products, vaccines and technologies?					3
NA					
VIII. To what extent are the good manufacturing practices the same for TCIM products and biomedicines?					5
Answer: Herbal medicine regulations impose the same standards for safety, efficacy and quality as for biomedicine.					
IX. To what extent does any national pharmacopoeia include both TCIM products and biomedicine products?					4
Answer: As of 2018, two herbal medicines were included on the country's essential medicines list.					
X. To what extent does any post-marketing surveillance or pharmacovigilance system include both conventional and TCIM products?					3
Answer: NA					
Average dimension score: 4					

A4.3 Country B

Traditional medicine in Country B is sometimes associated with spiritualism and a type of shamanism, especially in rural areas. From a regulatory perspective, Country B appears to emphasize the scientific rationale of traditional medicine and not the spiritual aspect. There is strong political will and commitment by the Government to integration of TCIM into its national health system, as indicated by the various political actions.

Fig. A4.3. Country B summary



Heath system building block		Indicators				
Leadership and governance	Dimension: decentralized vs centralized					
	Scoring System	1	2	3	4	5
	1. No national TCIM policy; generally, limited regulation or oversight of TCIM; TCIM practices are tolerated.					
	2. No TCIM national policy; little regulation or oversight of TCIM; TCIM practices are legal.					
	3. Neutral or not available (NA)					
	4. National TCIM policy; some regulation or oversight of TCIM					
	5. National TCIM policy; TCIM generally regulated					
	Indicators					
	I. Is there a national policy, law or regulation on TCIM? Answer: Regulations are in place for TCIM products, practices and practitioners.					4
	II. What are the extent and nature of government regulation of TCIM health-care practitioner and practices? Answer: Regulations are in place for TCIM products, practices and practitioners.					
	III. Is there a national office for TCIM? Answer: A specific programme supported by the Ministry of Health and other departments serves monitors implementation of policies and strategies for traditional medicine and coordinates intersectoral collaboration and the interface with other ministries and partners.					4
	Average dimension score: 4					
	Dimension: Pluralism vs unification					
	1. TCIM is recognized as fully distinct from biomedicine in any regulation, policy or framework.	1	2	3	4	5
	2. TCIM is usually recognized as distinct from biomedicine in any regulation, policy or framework.					
	3. Neutral or NA					
	4. TCIM is sometimes conceptualized as being blended with biomedicine in any regulation or policy.					
	5. TCIM is usually or fully conceptualized as blended with biomedicine in any regulation or policy.					
	Indicators					
	IV. To what extent are TCIM practices recognized as biomedical practices in any government policy or regulation? Answer: Appears to be pluralistic approach					2
	V. To what extent does any government regulation of health-care practitioners recognize TCIM practitioners as separate and distinct from biomedicine practitioners, or are TCIM practitioners and biomedicine practitioners regulated in a single blended system? Answer: Appears to be pluralistic approach					2
	VI. To what extent does any government regulation recognize TCIM health products as separate and distinct, or are they regulated in a single blended system with conventional medical products? Answer: N/A					3
	VII. To what extent does any government regulation recognize TCIM practitioner education as separate and distinct or is TCIM education regulated in a single blended system with conventional medical education? Answer: Appears to be pluralistic approach					2
	Average dimension score: 2.2					

Health system financing	Dimension: Decentralized vs centralized				
	Scoring System				
	1	2	3	4	5
	1. There is minimal core funding for TCIM.				
	2. There is minimal core funding for TCIM; supplementary or discretionary funding is available.				
	3. Neutral or NA				
	4. There is moderate core funding for TCIM.				
	5. TCIM is generally funded as part of the health-care package.				
	Indicators				
	I. To what extent is there funding for TCIM services, treatments and products?				
	Answer: The national health insurance scheme currently covers fees for diagnostic tests and consultations, including with a herbalist but not for other traditional medicine products or other services.				
	2				
	II. What is the nature and extent of coverage of TCIM practices by health insurance?				
	Answer: NA				
	3				
	III. To what extent are TCIM services, treatments and products paid for out of pocket by individuals?				
	Answer: Mostly				
2					
Average dimension score: 2.3					
Dimension: Pluralism vs unification					
Scoring system					
1	2	3	4	5	
1. Any TCIM funding is distinct.					
2. Any TCIM funding is usually distinct.					
3. Neutral or NA					
4. Any TCIM funding is sometimes blended with biomedicine funding.					
5. Any TCIM funding is usually or fully blended with biomedicine funding.					
Indicators					
IV. To what extent is any funding for TCIM distinct and separate or blended with funding for biomedicine practitioner services, treatments and products?					
Answer: Appears to be pluralistic					
2					
V. To what extent is any insurance coverage for TCIM practices distinct and separate or blended with funding for biomedicine practices?					
Answer: Appears to be pluralistic					
2					
Average dimension score: 2					

Health service delivery	Dimension: Decentralized vs centralized				
	Scoring system	1	2	3	4 5
	1. TCIM health services are not delivered as part of the government-supported health system.			3	
	2. TCIM health services are generally not delivered as part of the government-supported health system.				
	3. Neutral or NA				
	4. TCIM health services are generally delivered as part of the government-supported health system.				
	5. TCIM health services are delivered as part of the government-supported health system.				
	Indicators				
	I. How are TCIM health services delivered? Answer: Traditional health practitioners provide a substantial proportion of primary care but are not part of the health system.	1			
	II. How accessible are TCIM services to individuals throughout the country? Answer: Pilot projects with TCIM practitioners (mainly herbalists) in the Government-supported health system are under way but are not available to the majority of the population.	1			
	III. Consider the extent to which any quality assurance activities include TCIM health facilities. Answer: NA				
Average dimension score: 1.7					
	Dimension: Pluralism vs unification				
	Scoring system	1	2	3	4 5
	1. TCIM health services are distinct.				
	2. TCIM health services are usually distinct.				
	3. Neutral or NA				
	4. TCIM health services are sometimes blended with biomedicine health services.				
	5. TCIM health-care services are usually or fully blended with biomedicine services.				
	Indicators				
	IV. How are TCIM health services delivered (i.e. are they distinct services or blended with biomedicine health services?) Answer: There are some referrals between conventional doctors and medical herbalists but no joint consultations. Medical herbalists sometimes accompany conventional doctors on ward rounds and may occasionally be invited to in patient consultations. There is some conflict between conventional and traditional practitioner organizations, although there may be some cooperation at personal level.	1			
	V. To what extent do publicly funded health delivery facilities include TCIM? Answer: Pilot projects are under way to include TCIM practitioners (mainly herbalists) in the Government-supported health system; however, this is not available to the majority of the population.	1			
Average dimension score: 1					

Health workforce	Dimension: Decentralized vs centralized				
	Scoring system	1	2	3	4 5
	1. TCIM practice and education are not regulated.				
	2. TCIM practice and education are generally not regulated, and biomedicine practitioners may include some TCIM practices.				
	3. Neutral or NA				
	4. There is some oversight or regulation of TCIM practice and education.				
	5. TCIM practice and education are overseen and regulated by the government.				
	Indicators				
	I. To what extent do people have access to health services in the national and local health systems provided by a range of health-care providers, including TCIM practitioners? Answer: Traditional health practitioners provide a substantial proportion of primary care but are not part of the health system.	1			
	II. How much government oversight is there of the practices of TCIM health-care practitioners? Answer: An engaged federation of TCIM practitioner associations ensures the credibility and sanctity of practitioners and registers practitioners; however, many federation members are not registered and/or do not have a license to practise.	2			
	III. How much government oversight is there of the education and professional development of TCIM health-care practitioners? Answer: There is a federation of TCIM practitioner associations that is responsible for registration, licensing and regulation of traditional health practitioners and their practice.			4	
	Average dimension score: 2.3				
	Dimension: Pluralism vs unification				
	Scoring system	1	2	3	4 5
	1. TCIM and biomedicine practitioners have distinct and independent education and practices.				
	2. TCIM and biomedicine practitioners have distinct and independent education and practices but recognize the value of learning about each others' practices.				
	3. Neutral or NA				
	4. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap to some extent.				
	5. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap significantly.				
	Indicators				
	IV. How much overlap is there in the education of TCIM and biomedicine providers? Answer: Some biomedical knowledge may be integrated into some herbal medicine programmes.	2			
	V. How much overlap is there in the scope of practice of TCIM and biomedicine providers? Answer: There has been some conflict between biomedical and traditional medical practitioners' organization, although there is some degree of cooperation at a personal level.	1			
	Average dimension score: 1.5				

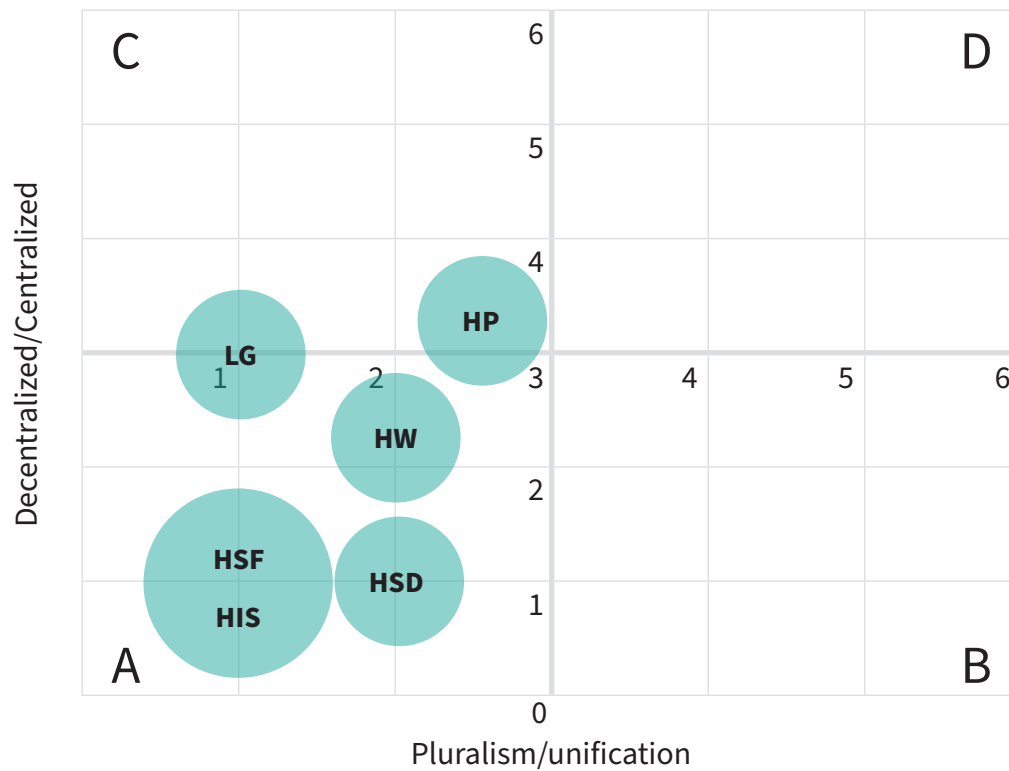
Health information systems	Dimension: Decentralized vs centralized				
	Scoring system	1	2	3	4 5
	1. TCIM and biomedicine practitioners sometimes voluntarily and informally share information.				
	2. TCIM and biomedicine practitioners regularly voluntarily share information.				
	3. Neutral or NA				
	4. TCIM and biomedicine practitioners are sometimes required to share information.				
	5. TCIM and biomedicine practitioners are usually required to share information.				
	Indicators				
	I. To what extent are TCIM biomedicine practitioners required to share information with each other?				
	Answer: Appears to be limited to Government intervention.	1			
Average dimension score: 1					
	Dimension: Pluralism vs unification				
	Scoring system	1	2	3	4 5
	1. TCIM and biomedicine practitioners have distinct, independent record-keeping practices.				
	2. TCIM and biomedicine practitioners have distinct record-keeping practices but recognize the value of coordinating information-sharing.				
	3. Neutral or NA				
	4. The record-keeping and information systems of TCIM and biomedicine practitioners share some requirements and facilitate some information-sharing.				
	5. The record-keeping and information systems of TCIM and biomedicine practitioners share many requirements and facilitate sharing of information.				
	Indicators				
	II. How much overlap is there in the billing, charting, referral, health data and information systems of TCIM and biomedicine providers?				
	Answer: Appears to be limited	1			
Average dimension score: 1					

Health products, vaccines and technologies	Dimension: Decentralized vs centralized					
	Scoring system		1	2	3	4 5
	1. No regulation of TCIM products.					
	2. Limited regulation of TCIM products					
	3. Neutral or NA					
	4. TCIM health products, vaccines and technologies are somewhat regulated by the Government.					
	5. TCIM health products and medical products are regulated.					
	Indicators					
	I. To what extent are TCIM health products, vaccines and technologies regulated by the government?					
	Answer: The Food and Drugs Authority regulates herbal products as well as conventional medical and herbal products.					4
	II. To what extent are the quality, safety, efficacy and cost-effectiveness of TCIM health products, vaccines and technologies assessed by the government?					
	Answer: Facilities in which traditional medicine practitioners manufacture traditional medicines are inspected by the Food and Drugs Authority.					4
	III. To what extent is the sale (who can sell and where) of TCIM products regulated?					
	Answer: The Food and Drugs Authority regulates the sale of both conventional medical and herbal products.					4
	IV. Does the national pharmacopeia include TCIM products such as herbal medicines?					
	As of 2018, two herbal medicines were included on the national essential medicines list.					
	V. Is there post-marketing surveillance or a pharmacovigilance system for TCIM products?					4
	Answer: NA					
					3	
	Average dimension score: 3.8					
	Dimension: Pluralism vs unification					
	Scoring system		1	2	3	4 5
	1. TCIM health products and technologies are distinctly and independently regulated, with distinct and independent routes of access.					
	2. TCIM health products and technologies are generally regulated distinctly and independently and are accessed by distinct and independent routes.					
	3. Neutral or NA					
	4. Regulation of and access to TCIM and biomedicine health products have shared requirements and pathways.					
	5. Regulation of and access to TCIM and biomedicine products and technologies overlap significantly with respect to shared requirements and pathways.					
	Indicators					
	VI. To what extent is there overlap in the regulation, quality control and adverse event reporting of TCIM health products, vaccines and technologies?					
	Answer: The standards for the safety, efficacy and quality of herbal medicines are the same as those for biomedicine.					5
	VII. To what extent is overlap in the way people access TCIM health products, vaccines and technologies?					
	NA				3	
	VIII. To what extent are the good manufacturing practices the same for TCIM products and biomedicines?					
	Answer: Herbal medicine regulations impose the same standards for safety, efficacy and quality as for biomedicine.					5
	IX. To what extent does any national pharmacopoeia include both TCIM products and biomedicine products?					
	Answer: As of 2018, two herbal medicines were included on the country's essential medicines list.					4
	X. To what extent does any post-marketing surveillance or pharmacovigilance system include both conventional and TCIM products?					
	Answer: NA				3	
	Average dimension score: 4					

A4.4. Country C

In Country C, traditional medicine is not regulated. It is learnt by oral tradition. A number of different types of complementary medicines are legalized in Country C including: homeopathy, naturopathy, and Traditional Chinese Medicine.

Fig. A4.4 Country C Summary



Heath system building block	Indicators					
Leadership and governance	Dimension: Decentralized vs centralized					
	Scoring system	1	2	3	4	5
	1. No TCIM national policy; generally limited regulation or oversight of TCIM; TCIM practices are tolerated					
	2. No TCIM national policy; little regulation or oversight of TCIM; TCIM practices are legal					
	3. Neutral or not available (NA)					
	4. National TCIM policy; some regulation or oversight of TCIM					
	5. National TCIM policy; TCIM generally regulated					
	Indicators					
	I. Is there a national policy, law or regulation on TCIM? Answer: A number of complementary therapies are regulated, including homeopathy, naturopathy, and traditional Chinese medicine); other practices are not regulated.					4
	II. What are the extent and nature of government regulation of TCIM health-care practitioners and practices? Answer: Traditional medicine practice is not regulated.			2		
	III. Is there a national office for TCIM? Answer: No				3	
Average dimension score: 3						
	Dimension: Pluralism vs unification					
	Scoring system	1	2	3	4	5
	1. TCIM is recognized as fully distinct from biomedicine in any regulation, policy or framework,					
	2. TCIM is usually recognized as distinct from biomedicine in any regulation, policy or framework.					
	3. Neutral or NA					
	4. TCIM is sometimes conceptualized as blended with biomedicine in any regulation or policy.					
	5. TCIM is usually or fully conceptualized as blended with biomedicine in any regulation or policy.					
	Indicators					
	IV. To what extent are TCIM practices recognized as distinct from biomedicine practices in any government policy or regulation? Answer: Regulated complementary therapies are legally independent and autonomous.			1		
	V. To what extent does any government regulation of health-care practitioners recognize TCIM practitioners as separate and distinct from biomedicine practitioners, or are TCIM practitioners and biomedicine practitioners regulated in a single, blended system? Answer: Professionals in the regulated complementary therapies are legally independent and autonomous. Practitioner associations are lobbying for each group to become autonomous and recognized as different professions, with individual registration systems, rules and regulations for practice and other aspects. The biomedicine group is considered to be opposed to complementary medicine groups.			1		
	VI. To what extent does any government regulation recognize TCIM health products as separate and distinct, or are they regulated in a single blended system with conventional medical products? Answer: TCIM health products are recognized as distinct.					1

	VII. To what extent does any government regulation recognize TCIM practitioner education as separate and distinct, or is TCIM education regulated in a single blended system with conventional medical education?					
	Answer: Education of professionals in regulated complementary therapies are legally independent and autonomous.	1				
	Average dimension score: 1					
Health system financing	Dimension: Decentralized vs centralized					
	Scoring system	1	2	3	4	5
	1. There is minimal core funding for TCIM.					
	2. There is minimal core funding for TCIM; supplementary or discretionary funding is available.					
	3. Neutral or NA					
	4. There is moderate core funding for TCIM.					
	5. TCIM is generally funded as part of the health-care package.					
	Indicators					
	I. To what extent is there funding for TCIM services, treatments and products?					
	Answer: Biomedicine is part of the national health service and is free. Complementary therapies are available only in private practice and are paid out of pocket. Some medical acupuncture is done in public hospitals by conventional doctors and is reimbursed.	1				
	II. What is the nature and extent of coverage of TCIM practices by health insurance?					
	Answer: Currently, only conventional pharmaceuticals are reimbursed, except for those available over the counter (including homeopathy). Traditional medicines, including food supplements, are not reimbursed. No insurance coverage is available for TCIM consultations.	1				
	III. To what extent are TCIM services, treatments and products paid out of pocket by individuals?					
	Answer: See above	1				
	Average dimension score: 1					
	Dimension: Pluralism vs unification					
	Scoring system	1	2	3	4	5
	1. Any TCIM funding is distinct.					
	2. Any TCIM funding is usually distinct.					
	3. Neutral or NA					
	4. Any TCIM funding is sometimes blended with biomedicine funding.					
	5. Any TCIM funding is usually or fully blended with biomedicine funding.					
	Indicators					
	IV. To what extent is any funding for TCIM distinct and separate or blended with funding for biomedicine practitioner services, treatments and products?					
	Answer: In some cases, medical acupuncture conducted in public hospitals by conventional doctors is reimbursed. Most TCIM is paid out of pocket by the patient.	1				
	V. To what extent is any insurance coverage for TCIM practices distinct and separate or blended with funding for biomedicine practices?					
	Answer: Insurance coverage for TCIM practices is distinct.	1				
	Average dimension score: 1					

Health service delivery	Dimension: Decentralized vs centralized					
	Scoring system	1	2	3	4	5
	1. TCIM health services are not delivered as part of the government supported health-care system.					
	2. TCIM health services are generally not delivered as part of the government-supported health system.					
	3. Neutral or NA					
	4. TCIM health services are generally delivered as part of the government-supported health system.					
	5. TCIM health services are delivered as part of the government-supported health system.					
	Indicators					
	I. How are TCIM health services delivered?					
	Answer: Administrators in the health system hire health-care professionals for public hospitals; there are no TCIM practitioners in public hospitals.	1				
	II. How accessible are TCIM services to individuals throughout the country?					
	Answer: See above.	1				
	III. Consider the extent to which any quality assurance activities include TCIM health facilities.					
	Answer: See above.	1				
	Average dimension score: 1					
	Dimension: Pluralism vs unification					
	Scoring system	1	2	3	4	5
	1. TCIM health services are distinct.					
	2. TCIM health services are usually distinct.					
	3. Neutral or NA					
	4. TCIM health services are sometimes blended with biomedicine services.					
	5. TCIM health-care services are usually or fully blended with biomedicine services.					
	Indicators					
	IV. How are TCIM health services delivered (are they distinct services or blended with biomedicine health services?)					
	Answer: In a few State hospitals, some nurses and medical doctors deliver medical acupuncture, and some private hospitals have TCIM professionals; however, they are not perceived as working together in an integrative way.	2				
	V. To what extent do publicly funded health delivery facilities offer TCIM?					
	Answer: See above	2				
	Average dimension score: 2					

Health workforce	Dimension: Decentralized vs centralized				
	Scoring system				
	1.	2.	3.	4.	5.
	1. TCIM practice and education are not regulated.				
	2. TCIM practice and education are generally not regulated, and biomedicine practitioners may incorporate some TCIM practices.				
	3. Neutral or NA				
	4. There is some oversight or regulation of TCIM practice and education.				
	5. TCIM practice and education are overseen and regulated by the government.				
	Indicators				
	I.	To what extent do people have access to health services and care in the national and local health systems provided by a range of health care providers, including TCIM practitioners?			
		Answer: The health system hires health-care professionals for public hospitals within the national health service, which currently does not cover practitioners of Traditional medicine.			
	II.	How much government oversight (e.g. regulations) is there of the practices of TCIM health-care practitioners?			
		Answer: A number of complementary medicine practices are regulated; others, such as traditional medicine, are not.			
	III.	How much government oversight is there of the education and professional development of TCIM health-care practitioners?			
		Answer: Biomedicine and complementary medicine practitioners are regulated by the same body. Traditional medicine is not regulated and is learnt by oral tradition			
	Average dimension score: 2.3				
	Dimension: Pluralism vs unification				
	Scoring system				
	1.	2.	3.	4.	5.
	1. TCIM and biomedicine practitioners have distinct and independent education and practices.				
2. TCIM and biomedicine practitioners have distinct and independent education and practices but recognize the value of learning about each others' practices.					
3. Neutral or NA					
4. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap to some extent.					
5. The educational requirements and scopes of practice of TCIM and biomedicine practitioners overlap significantly.					
Indicators					
IV.	How much overlap is there in the education of TCIM and biomedicine providers?				
	Answer: Complementary medicine practitioners are independent and autonomous professionals. Conventional doctors are authorized to perform medical acupuncture.				
V.	How much overlap is there in the scope of practice of TCIM and biomedicine providers?				
	Answer: See above				
Average dimension score: 2					

Health information systems	Dimension: Decentralized vs centralized					
	Scoring system	1	2	3	4	5
	1. TCIM and biomedicine practitioners sometimes voluntarily and informally share information.					
	2. TCIM and biomedicine practitioners regularly voluntarily share information.					
	3. Neutral or NA					
	4. TCIM and biomedicine practitioners are sometimes required to share information.					
	5. TCIM and biomedicine practitioners are usually required to share information.					
	Indicators					
	I. To what extent are TCIM biomedicine practitioners required to share information with each other?					
	Answer: Not required					
Average dimension score: 1						
	Dimension: Pluralism vs unification					
	Scoring system	1	2	3	4	5
	1. TCIM and biomedicine practitioners have distinct, independent record-keeping practices.					
	2. TCIM and biomedicine practitioners have distinct record-keeping practices but recognize the value of sharing information with others.					
	3. Neutral or NA					
	4. The record-keeping and information systems of TCIM and biomedicine practitioners share some requirements and facilitate some information sharing.					
	5. The record-keeping and information systems of TCIM and biomedicine practitioners share many requirements and facilitate sharing of information.					
	Indicators					
	II. How much overlap is there in the billing, charting, referral, health data and information systems of TCIM and biomedicine providers?					
	Answer: None					
	Average dimension score: 1					

Health products, vaccines and technologies	Dimension: Decentralized vs centralized					
	Scoring system	1	2	3	4	5
	1. No regulation of TCIM products					
	2. Limited regulation of TCIM products					
	3. Neutral or NA					
	4. TCIM health products, vaccines and technologies are regulated to some extent by the government.					
	5. TCIM health products and medical products are regulated.					
	Indicators					
	I. To what extent are TCIM health products, vaccines and technologies regulated by the government?					
	Answer: A government agency under the Health Ministry evaluates, authorizes, regulates and controls biomedicines, health products and homeopathic and herbal medicines.					4
	II. To what extent are the quality, safety, efficacy and cost-effectiveness of TCIM health products and technologies assessed by the government?					
	Answer: See above					3
	III. To what extent is the sale (who can sell and where) of TCIM products regulated?					
	Answer: NA					3
	IV. Does the national pharmacopeia include TCIM products such as herbal medicines?					
	Answer: NA					3
	V. Is there post-marketing surveillance or a pharmacovigilance system for TCIM products?					
	Answer: NA					3
	Average dimension score: 3.2					
	Dimension: Pluralism vs unification					
	Scoring system	1	2	3	4	5
	1. TCIM health products, vaccines and technologies are distinctly and independently regulated, with distinct and independent routes of access.					
	2. TCIM health products and technologies are generally regulated distinctly and independently and are accessed by distinct and independent routes.					
	3. Neutral or NA					
	4. Regulation of and access to TCIM and biomedicine health products have shared requirements and pathways.					
	5. Regulation of and access to TCIM and biomedicine products and technologies overlap significantly with respect to shared requirements and pathways.					
	Indicators					
	VI. To what extent is there overlap in the regulation, quality control and adverse event reporting for TCIM health products, vaccines and technologies?					
	Answer: NA					3
	VII. To what extent is there overlap in how people access TCIM health products, vaccines and technologies?					
	Answer: Limited overlap	1				
	VIII. To what extent are any good manufacturing practices the same for TCIM products and biomedicines?					
	Answer: NA					3
	IX. To what extent does the national pharmacopoeia include both TCIM products and biomedicine products?					
	Answer: NA					3
	X. To what extent does any post-marketing surveillance or pharmacovigilance system include both conventional and TCIM products?					
	Answer: NA					3
	Average dimension score: 2.6					

Annex 5. Details of the countries studied, selected and visited

Countries studied

In consultation with the regional advisors on TCIM and the country offices, WHO prepared an outline of the methodology to study the role of TCIM and the state of integration in national health systems. WHO requested Member States to share relevant information that would allow us to understand the current role and position of TCIM in their health systems.

The information sought from Member States includes:

- national policy for TCIM
- national laws and specific regulations
- available healthcare facilities
- available healthcare professional organizations
- available education and training options
- registered patient experience with TCIM
- summary of the health insurance

WHO received responses from 20 Member States, and their national policies and information materials were studied to inform visit in selected countries.

Countries selected

The patterns of use of TCIM presented in the WHO Traditional Medicine Strategy 2014–2023 (3) were used to identify Member States that represented a range of approaches to integration of TCIM into their health systems.

The first pattern of use of TCIM was found in countries in which TCIM is a primary source of health care, in the African Region, where traditional medicine is firmly rooted, although there is limited formal documentation and education. Given the large number of Member States in the African Region, and their diversity, four Member States were contacted to represent the region. These were Ghana and Mali from the Economic Community of West African States (ECOWAS) and Madagascar and South Africa from the Southern African Development Community (SADC).

The second category of countries was in which TCIM is widespread because of cultural and historical influences but in which there is still scope for integration at systemic level. These included countries in the South-East Asia and Western Pacific regions, where, for example, the Ayurveda system of traditional medicine in India and traditional Chinese medicine in China are globally practised systems of traditional medicine. China, India, Japan, Republic of Korea and Thailand, with the largest populations, were approached to identify best practices and areas in which there could be improvement.

The third pattern of TCIM use was identified in countries in which the structure of the health system is usually well developed, TCIM originates mainly from the outside and which had attempted to integrate complementary medicine into their health systems. Case studies included Brazil, Norway and Portugal.

Countries visited

To supplement the secondary research, an on-site visit was planned to 3 selected countries representing each above-mentioned category. The objectives of the visits are:

- (a) To conduct a review of the traditional and complementary medicine (TCIM) situation in the country covering areas of policy and regulation, TCIM products, practices and practitioners, education, research and service delivery;
- (b) To learn about the existing situation of integration of TCIM into the national health systems and current ground realities.

These visits allowed the team to observe integration at the various levels of service delivery and findings contributed to subsequent development of the first draft of the document.

Annex 6. Participants in the online consultation

African Region

Dr Ossy M. J Kasilo, *Former* Regional adviser for Traditional Medicine, World Health Organization, Republic of Congo

Region of the Americas

Prof Heather Boon, Professor and Vice-Provost, Faculty & Academic Life, University of Toronto, Canada

Dr Daniel Gallego-Perez, Integrative, Complementary, and Traditional Health Practices (ICTHP), Colombia

Dr Sara L Warber, Clinical Professor Emerita, Family Medicine, University of Michigan, United States

South-East Asia Region

Dr Rajeshwari Singh, Ministry of AYUSH, India

European Region

Prof Vinjar Fonnebo, *Former* Director National Research Center in Complementary and Alternative Medicine, UiT The Arctic University of Norway, Norway

Dr Dheepa Rajan, European Observatory on Health Systems and Policies, Switzerland

Dr Qi Zhang, Expert in Traditional, Complementary and Integrative Medicine, *Former* Unit Head of TCI, Department of Integrated Health Services, WHO, Geneva, Switzerland

Eastern Mediterranean Region

Dr Mohamed Khalil, Consultant, Public Health & Research Development, Cairo, Egypt

Western Pacific Region

Dr Rachel Canaway, Health & Biomedical Research Information Technology Unit, Department of General Practice & Primary Care, The University of Melbourne, Australia

Dr Peter Bai James, National Centre for Naturopathic Medicine, Faculty of Health, Lismore, New South Wales, Australia

WHO Secretariat

Dr Rudi Eggers, Director, Department of Integrated Health Services

Dr Sungchol Kim, Unit Head of Traditional, Complementary and Integrative Medicine

Dr Qin Liu, Technical Officer, Unit of Traditional, Complementary and Integrative Medicine

Dr Aditi Sharan, Consultant, Unit of Traditional, Complementary and Integrative Medicine

Dr, Geetha Krishnan Gopalakrishna Pillai, Global Traditional Medicine Centre, Jamnagar, Gujarat, India

Annex 7. Summary of the Scoping Review for Integration of TCIM into Health Systems

Traditional, complementary and integrative medicine (TCIM) is practiced globally and many countries are struggling with how to incorporate TCIM as part of their health systems. The objective of the scoping review was to identify key concepts and research conducted on the integration of traditional & complementary medicine into health systems around the world to inform the proposed World Health Organization (WHO) guidance document.

The primary review question was: **What are the key elements used to define the integration of traditional & complementary medicine into health systems?** In addition, the review addressed the sub-question: **How are the levels of integration of traditional & complementary medicine into health systems described, differentiated and assessed?**

Six electronic databases were searched to identify published research:

- Ovid Medline (1946-present including Epub ahead of print, in-process, and other non-indexed citations)
- Ovid Embase (1947-present)
- Ovid AMED (Allied and Complementary Medicine) 1985 – present
- EBSCO CINAHL Plus (1981-present)
- LILACS (Latin American and Caribbean Health Sciences Literature)
- Elsevier Scopus

The search strategy was translated into each database using that platform's command language including title, abstract, author provided keywords, controlled vocabulary, and subject headings when possible. No date or language limits were used by an academic health sciences librarian to identify relevant published research. This was supplemented by a grey literature search and a review of the reference lists of studies meeting the inclusion criteria. Each article was independently assessed by two of the four reviewers using Covidence software in two stages: first, titles and abstract screening; and second, screening based on a review of the full text. Any discrepancies were resolved by the lead author and discussion with the other reviewers if needed.

A method for conducting qualitative secondary thematic analysis called Best Fit Framework Analysis was used to synthesize the evidence. The WHO Health System Building Blocks (*Leadership and Governance; Health System Financing; Health Service Delivery; Health Workforce; Health Information Systems; Health Products, Vaccines & Technologies*) were used as the theoretical framework to inform the development of the original qualitative codes. NVIVO software was used to facilitate the qualitative analysis.

A total of 25,512 articles were identified for this review and after screening, 53 were found to meet the inclusion criteria. The articles included were published between the years 1976 and 2023 and were written by authors based in all six WHO regions. Less than half (n=21; 40%)

of the articles provided a formal definition of integration of TCIM into the health system and each article provided a unique definition. Almost half (n=24; 45%) of the articles identified frameworks that described different levels or models of integration. The number of levels or models of integration identified within each article ranged from three to seven. No single model was identified as likely to be applicable or “best” in all contexts.

This scoping review identified two key dimensions that differentiate ways in which TCIM are integrated into health systems: i) Decentralized vs Centralized Approach and ii) Pluralism vs Unification. Based on these dimensions, we identified four Categories of models of integration: Category A (Patient-led) which are decentralized and pluralistic in nature; Category B (Practitioner-led) which are decentralized and blended; Category C (Coordinated System) which are centralized and pluralistic; and Category D (Blended System) which are centralized and blended.

Eleven (21%) articles provided examples of how to assess the level or model of integration of a health system or part thereof; however, many of the assessment approaches were relatively undeveloped. There were two main approaches taken to assessment: i) relying on general principles and success factors (e.g., governance, financial incentives); and ii) focusing on micro-level criteria (e.g., location of integrative practitioner’s room; number of collaboration meetings).

In conclusion, given the wide variety of country contexts that shape health system designs, there was no “one size fits all” or “best” model of integration of TCIM into health systems identified. The majority of the models described in the literature focused on the health system building block *health service delivery* with relatively less attention paid to the other WHO health system building blocks. We identified a need for more research on the measurement and/or assessment of integration type or level.

